



POPULATION-BASED TEMPLATE

Optimal care pathway
for [insert population group] with cancer

X edition

Development draft: [Add version number]

Publication date: [XX Month 20XX]

Endorsed by:

Add endorsing
organisation logo/s

Instructions for developers

This template has been created to help developers of population-based Optimal Care Pathways (OCPs) develop a high-quality and evidence-based resource that is consistent with the library of existing pathways. The template mirrors the core content of the cancer-specific OCP template and provides space to add considerations for a particular population group relevant to the Principles for Optimal Cancer Care [<hyperlink to be added>](#) and each pathway step. It contains two types of content:

- **core content** that is standard across all OCPs and cannot be changed
- **editable content** that must be completed or adapted for your population group.

Tips for success

- Follow these instructions when developing your OCP. If you have any questions, contact Cancer Australia at OCPs@canceraustralia.gov.au
- Keep content high level and scannable. Avoid lengthy narrative paragraphs.
- Review the Principles for Optimal Cancer Care [<hyperlink to be added>](#) when developing content to avoid duplication and identify where cross-referencing is appropriate.

Types of content in this template

- **Black text** is core content that cannot be edited. This is standard content across all OCPs.
- Text **[highlighted in yellow]** indicates words to be added / adapted for this population group / OCP.
- **Purple boxes** indicate content requiring developer input specific to this population group / OCP. Instructions are provided in **purple text**.
- **Key considerations tables** contain brief statements summarising what optimal care looks like at each step of the cancer pathway. Follow the **purple prompts** to add population-specific considerations as new rows in these tables.
- **Checklists** are actionable items for health professionals at key decision points. Follow the **purple prompts** to add any population-specific actions as new rows in these lists.
- **Supportive care sections:** Each step includes supportive care considerations. Follow the **purple prompts** to add population-specific content.
- **'Find out more' boxes** are curated links to guidelines and resources. These will appear on the digital hub with links from relevant sections. Follow the **purple prompts** to add any specific resources for this population group and include the associated hyperlink.

What you can and cannot change

- **Numbered headings and subheadings cannot be changed or reordered.** This ensures consistency across the OCP library and aids with digitisation.
- Additional **third-level subheadings (i.e. non-numbered headings) can be added within sections.** Consider additional sub-headings sparingly.
- Sections **can be removed where content is not relevant** to this population group.

Making changes

If you identify content that you believe should be added as core information to the template (e.g. a new section, consideration or action relevant to all population-based OCPs), note this separately with Cancer Australia. Developers cannot edit core template content directly. Suggestions will be considered as part of the centralised review process.

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Preface

Statement of acknowledgement

We acknowledge Aboriginal and Torres Strait Islander people as the Traditional Custodians of Country throughout Australia. We pay our respects to Elders, past and present.

We celebrate the ongoing connections of Aboriginal and Torres Strait Islander peoples to Country, culture, community, family and tradition and recognise these as integral to health, healing and wellbeing.

We acknowledge great diversity among Aboriginal and Torres Strait Islander peoples, and the contribution of the many voices, knowledge systems and experiences that guide all efforts to create a culturally safe and responsive cancer system that is equitable to all.

Endorsement

This optimal care pathway was formally endorsed by the Cancer and Population Screening Committee (CAPS) following advice from Cancer Australia's National Cancer Expert Group (NCEG). Formal endorsement by the CAPS Committee acknowledges the importance of this Optimal Care Pathway and the responsibility of the health system to deliver care in an appropriate, culturally safe, responsive and coordinated manner to ensure access and that optimal health outcomes are experienced by all Australians.

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Enquiries about this publication can be sent to OCPs@canceraustralia.org.au

About Optimal Care Pathways

Optimal Care Pathways (OCPs) are nationally endorsed evidence- and consensus-based guides to best practice care across the cancer continuum. They set a national standard for consistent, high-quality cancer care for everyone in Australia, regardless of who they are or where they live.

OCPs are embedded within the [Australian Cancer Plan](#) as national standards for delivery of high-quality, consistent cancer care. Their development is guided by the [National Optimal Care Pathways Framework](#), which provides a consistent approach to prioritisation, development, update and endorsement of OCPs and supports their adoption into health systems.

Intent of the Optimal Care Pathways

Map nationally agreed best practice approaches to cancer care, placing the person with cancer at the centre of care.

Set the standard for quality cancer care in Australia by describing what optimal care looks like across the cancer continuum.

Reduce unwarranted variation in access, care and outcomes by providing a nationally consistent approach to benchmark and improve services and practice.

Guide service improvement by helping health services identify gaps and opportunities to achieve optimal care.

Support clinician practice by giving health professionals evidence-based information, checklists and tools to understand and deliver best practice care.

Inform and empower people affected by cancer to have informed conversations with their healthcare teams about optimal care and what they should expect.

Drive equity by promoting accessible, culturally safe and responsive care that reduces unwarranted variations in access, care and outcomes.

Types of Optimal Care Pathways

Cancer-specific OCPs

Provide guidance on prevention, early detection, diagnosis, treatment and care of specific cancer types, including recommended timeframes.

Population-based OCPs

Provide guidance on considerations across the cancer continuum for priority population groups who have distinct care needs.

Cancer-specific and population-based OCPs can be used separately or together to create a **tailored pathway based on individual needs.**

All OCPs are underpinned by **[Principles for Optimal Cancer Care](#)**. <hyperlink to be added>

Using Optimal Care Pathways

OCPs are **designed for use by all those involved in cancer care.**



Health professionals



**Health service administrators,
managers and policy makers**



**People with cancer, their
carers and families**

OCPs are designed to be used with clinical guidelines and eviQ protocols as part of a **comprehensive approach to cancer care.**

Optimal Care Pathways

Nationally endorsed cancer care pathways with standards of cancer care that everyone with cancer in Australia should expect

Clinical practice guidelines

Systematically developed evidence-based statements and recommendations to support decision-making in specific clinical circumstances

eviQ protocols

Evidence-based, consensus driven cancer treatment protocols to guide use of specific treatments or interventions at point of care

OCPs do not constitute medical advice or replace clinical judgement. They provide links to relevant guidelines, protocols and other resources.

Pathway resources

Each OCP is available as a suite of three complementary resources, designed for different users and purposes:



Full OCP



OCP Quick Reference Guide



**OCP Consumer Guide:
*Guide to best practice
cancer care***

Comprehensive evidence- and consensus-based guide for health professionals and health services

Summary of the full OCP for use at point of care

Plain-language summary for people affected by cancer, their carers and families
Available in multiple languages

Principles for Optimal Cancer Care

The Principles for Optimal Cancer Care set the foundations for delivery of information, care and support for everyone who is at risk of and affected by cancer. Everyone in the healthcare system is responsible for delivering cancer care aligned to the Principles.

Principles are relevant to all cancer types and all populations. Population-based OCPs may identify unique considerations for implementation for a particular population group.



Equity <Link to Principle> <hyperlink to be added>

Health equity in cancer outcomes is the absence of disparities for people with different levels of social advantage.¹



Person-centred care <Link to Principle> <hyperlink to be added>

Person-centred care means informing and involving people in their care, and respecting and responding to their individual needs, preferences and values, as well as those of their families and carers.²



Safe and quality care <Link to Principle> <hyperlink to be added>

Safe and quality care means delivering the most appropriate and best-value care, while keeping people safe from preventable harm in the delivery of that care.³



Multidisciplinary care <Link to Principle> <hyperlink to be added>

Multidisciplinary care is an integrated team approach that involves all relevant health professionals discussing all relevant treatment options and making joint recommendations about treatment and supportive care plans that consider the individual characteristics and preferences of the person with cancer.⁴



Supportive care <Link to Principle> <hyperlink to be added>

Supportive care involves prevention and management of the adverse effects of cancer and its treatment, including management of physical and psychological symptoms and side effects across the cancer continuum. Supportive care aims to improve the quality of rehabilitation, secondary cancer prevention, survivorship, and end-of-life care.⁵



Navigation and care coordination <Link to Principle> <hyperlink to be added>

Navigation and care coordination are related person-centred interventions that aim to empower patients, families and carers affected by cancer to facilitate safe and timely access to optimal cancer-related care and support.⁶



Communication <Link to Principle> <hyperlink to be added>

Good communication is an essential part of cancer care and a responsibility of everyone involved in health care across the cancer continuum.⁷



Research and clinical trials <Link to Principle> <hyperlink to be added>

Research and clinical trials generate evidence to improve understanding of cancer and to accelerate progress in prevention, detection, diagnosis, treatment, supportive care, survivorship care, palliative and end-of-life care.⁸

Population-based considerations for the Principles for Optimal Cancer Care



<Optional> Review the Principles for Optimal Cancer Care [<hyperlink to be added>](#)

Use this section to add content to each Principle specific to this population group; Principles should be high-level and apply to all cancer types. Use the style and format of the existing Principle content (bullet points, checklists and links to relevant resources); Avoid long paragraphs of narrative text; Include references as Endnotes. Do not duplicate content already in the Principles; Do not duplicate contextual information included in introductory scope and context sections of the OCP for this population group or in pathway steps.

Equity

[<hyperlink to be added>](#) For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about equity specific to this population group; use subheadings and new rows as appropriate.

Population-based considerations for equity

Key considerations relating to equity in cancer care for [population group] include:

- *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for equitable cancer care

Key actions for delivery of equitable cancer care for [population group] include:

- ✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about equity for this population group, including [hyperlink](#)

Person-centred care

[<hyperlink to be added>](#) For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about person-centred care specific to this population group; use subheadings and new rows as appropriate.

Population-based considerations for person-centred care

Key considerations relating to person-centred care for [population group] include:

- *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for person-centred cancer care

Key actions for delivery of person-centred cancer care for [population group] include:

- ✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about person-centred care for this population group, including hyperlink

Safe and quality care

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about safe and quality care specific to this population group; use subheadings and new rows as appropriate

Population-based considerations for safe and quality care

Key considerations relating to safe and quality care for [population group] include:

- *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for safe and quality care

Key actions for delivery of safe and quality care for [population group] include:

- ✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about safe and quality care for this population group, including hyperlink

Multidisciplinary care

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about multidisciplinary care specific to this population group; refer to the content about multidisciplinary teams and meetings in pathway steps 3, 4 and 6 to avoid duplication; use subheadings and new rows as appropriate

Population-based considerations for multidisciplinary care

Key considerations relating to multidisciplinary care for [population group] include:

- *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for multidisciplinary care

Key actions relating to multidisciplinary care for [population group] include:

- ✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about multidisciplinary care for this population group, including hyperlink

Supportive care

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care specific to this population group; refer to the supportive care content at each pathway step to avoid duplication; use subheadings and new rows as appropriate

Population-based considerations for supportive care

Key considerations relating to supportive care for [population group] include:

- *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for supportive care

Key actions relating to supportive care for [population group] include:

- ✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about supportive care for this population group, including hyperlink

Navigation and care coordination

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about navigation and care coordination specific to this population group; refer to the supportive care content at each pathway step to avoid duplication; use subheadings and new rows as appropriate

Population-based considerations for navigation and care coordination

Key considerations relating to navigation and care coordination for [population group] include:

- insert key considerations as bullets or short paragraphs with sub-headings



Population-based actions for navigation and care coordination

Key actions relating to navigation and care coordination for [population group] include:

- ✓ insert key actions; use a new row for each action



Find out more

<Optional> Add resources about navigation and care coordination for this population group, including hyperlink

Communication

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about communication specific to this population group; refer to the supportive care content at each pathway step to avoid duplication; use subheadings and new rows as appropriate

Population-based considerations for communication

Key considerations relating to communication for [population group] include:

- insert key considerations as bullets or short paragraphs with sub-headings



Population-based actions for communication

Key actions relating to communication for [population group] include:

✓ *insert key actions; use a new row for each action*



Find out more

<Optional> Add resources about communication for this population group, including hyperlink

Research and clinical trials

<hyperlink to be added> For consultation purposes, please refer to Attachment A: Principles



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about research and clinical trials specific to this population group; use subheadings and new rows as appropriate

Population-based considerations for research and clinical trials

Key considerations relating to research and clinical trials for [population group] include:

• *insert key considerations as bullets or short paragraphs with sub-headings*



Population-based actions for research and clinical trials

Key actions relating to research and clinical trials for [population group] include:

✓ *insert key actions; use a new row for each action*



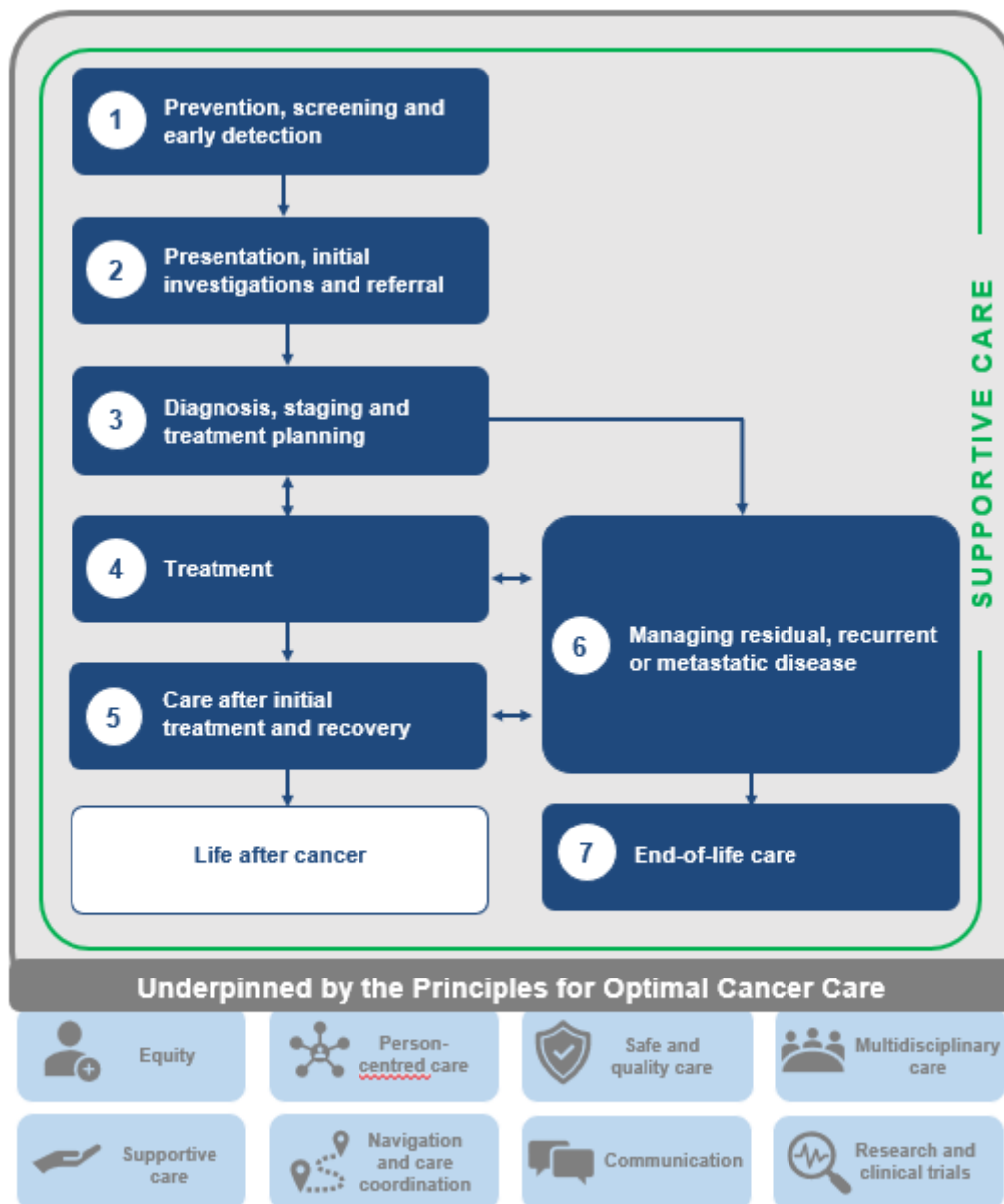
Find out more

<Optional> Add resources about research and clinical trials for this population group, including hyperlink

Steps of the Optimal Care Pathway

Optimal Care Pathways (OCPs) outline nationally agreed best practice across the entire cancer continuum. While the pathway steps are numbered in sequence, this does not mean the OCPs have to be used in a linear way. Optimal care for an individual is shaped by their cancer type, stage of disease and a range of personal characteristics and circumstances.

Supportive care is a critical pathway-spanning step for the individual and their families and carers.



Optimal care is not always linear and is shaped by cancer type, stage of disease, and the person's circumstances, needs and preferences.

About this Optimal Care Pathway

Scope



Complete the boxes

Edit highlighted content to provide a concise overview of what this OCP covers and who it is for; delete content that is not relevant

This Optimal Care Pathway (OCP) provides guidance for the care of [population group] with cancer, from prevention and early detection through to survivorship or end-of-life. It is designed to be used alongside the relevant cancer-specific OCP to tailor care to the individual.

The OCP applies to [population group], defined for the purposes of this pathway as [insert how this population group is defined e.g. age range, characteristics or circumstances].

<Optional> List people or circumstances explicitly excluded from this definition

This pathway does not address [insert anything explicitly out of scope for this population group, e.g. specific settings, conditions or age boundaries].

This OCP should be considered together with relevant cancer-specific and other relevant population-based OCP(s) if required to create a tailored pathway that meets the needs of the individual.

About this population group

Overview of the population

 Complete the boxes	
<i>Briefly describe this population group (use current Australian data)</i>	
Size and demographics	• Describe the size and key demographic characteristics of this population group
Cancer burden	• Describe what is known about cancer incidence, mortality, survival and burden of disease in this population group, including disparities compared with the general population; Use most current AIHW data
Variation within the population	• List any significant variation in cancer statistics within this population group if known (e.g. by age, sex, geography, or intersecting priority population groups); •Use most current AIHW data
Trends	• Describe changes over time in the size of this population group or in cancer risk or outcomes for the group, and drivers of these trends if known

Key considerations for delivery of optimal cancer care

 Complete the box	
<i>Briefly describe key considerations for delivery of optimal cancer care to this population group</i>	
• Provide a high-level summary of unique needs, barriers and considerations that shape delivery of optimal care for this population group across the cancer continuum • You will be prompted to expand on these high-level considerations at relevant pathway steps • Include: – key factors that influence access to, and experience of, cancer care for this population group – recent or near-horizon changes influencing how care for this population group is organised – key organisations, services, programs or models of care specific to this population group in Australia • Do not duplicate overarching content for the Principles for Optimal Cancer Care – use the ‘See also’ box (example below) to cross reference relevant Principles for further information	
 See also	
• <Optional> List relevant Principle(s)	

Step 1: Prevention, screening and early detection

Best practice guidance for reducing the risk of cancer and supporting early detection, including screening, surveillance and symptom awareness.

1.1 Prevention

Preventive health and health promotion are essential to improving health and quality of life.⁹

Advice about modifiable risk factors for cancer should be considered alongside social, environmental, structural, economic, cultural, biomedical, commercial and digital determinants of health.¹⁰

1.1.1 Risk factors

Common¹ risk factors for cancer include:^{11,12}

- smoking tobacco
- alcohol use above recommended Australian guidelines¹³
- ultraviolet (UV) radiation exposure
- low physical activity
- being overweight or obese
- exposure to oncoviruses or carcinogens.



Complete the appropriate box

Population-based risk factors for cancer

- *List factors influencing risk of cancer specific to this population group as bullet points*
- *For example, a higher prevalence of a known risk factor, factors influencing awareness of risk or likelihood of reporting or detecting risk in this population group*

Or

There are no known factors influencing risk of cancer specific to [population group].

1.1.2 Risk reduction strategies

Health professionals should opportunistically discuss and address modifiable risk factors at every appropriate clinical contact, providing guidance that includes:¹⁴

- stopping smoking
- stopping or reducing alcohol use
- achieving and maintaining a healthy weight
- making changes to diet and physical activity
- adopting sun safe behaviour
- maintaining relevant vaccinations.

¹ Not all risk factors are relevant for all cancer types



Complete the appropriate box

Population-based recommendations for reducing cancer risk

Recommendations to reduce the risk of cancer in [population group] include:

- *list evidence-based guidance for reduction of cancer risk specific to this population group*
- *use a new bullet point for each recommendation*

Or

There are no specific considerations for reducing the risk of cancer in [population group].



Find out more

[Resources on risk reduction for cancer](#)

<Optional> Add resources about reducing cancer risk specific to this population group, including hyperlink

1.1.3 Genetic testing for inherited (germline) cancer risk

Some cancers are caused or substantially influenced by inherited genetic variants. Identifying people with an elevated familial or genetic risk enables targeted surveillance and, in some cases, risk-reducing interventions.¹⁵ In addition to testing the individual, cascade testing can enhance the benefits of genetic testing by empowering family members to undertake surveillance and risk-reducing interventions.

Predictive genetic testing should be considered in individuals with:

- a known pathogenic variant identified in their family
- a significant family history of cancer, where genetic testing of previously affected family members has not been possible.¹⁶

Predictive genetic testing may be coordinated by a clinical genetics service or familial cancer service. It requires specific consent and genetic counselling before and after the test. Cancer services should have clear referral pathways to genetics / familial cancer services.



Complete the appropriate box

Population-based considerations for genetic risk and testing

Considerations for genetic risk and testing in [population group] include:

- *list known considerations for inherited cancer risk / genetic testing specific to this population group*
- *for example, genetic variants with a higher prevalence in this population, access to genetic testing / counselling and cultural factors influencing openness to discuss genetic testing and inherited risk*

Or

There are no specific considerations for genetic testing in [population group].



Find out more

[Resources on genetic testing for cancer](#)

<Optional> Add resources about hereditary risk specific to this population group, including hyperlink

1.2 Early detection

Early detection encompasses both population-based screening of asymptomatic individuals and enhanced surveillance for those at elevated risk. Being aware of the risk factors and who is at high risk can guide monitoring and referral to a cancer specialist.

Finding cancer at an early stage, or identifying pre-malignant conditions, significantly improves treatment options and outcomes.

1.2.1 Population-based screening recommendations



Complete the appropriate box

List considerations for participation in population-based cancer screening programs (BreastScreen Australia, National Cervical Screening Program, National Bowel Cancer Screening Program, National Lung Cancer Screening Program) specific to this population group)

Population-specific considerations for cancer screening

Factors influencing awareness and participation of [population group] in population-based cancer screening programs include:

- *list considerations relevant to all screening programs and those specific to a particular screening program; use a different row and sub-heading if considerations apply to one specific program*
- *consider including recommended actions to help promote or encourage participation in screening for this population group*

Or

There are no specific considerations for population-based cancer screening in [population group].

1.2.2 Surveillance recommendations



Complete the appropriate box

Population-specific surveillance considerations

Factors that may influence cancer surveillance in [population group] include:

- *list considerations for surveillance specific to this population group*

Or

There are no specific considerations for cancer surveillance in [population group].



Find out more

<Optional> *Add relevant guidelines or information resources about population-based screening or surveillance specific to this population group*

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Communication checklist when discussing cancer risk and early detection

- ✓ Provide information about risk and behaviour change in a non-judgemental way
- ✓ Provide information about risk, behaviour change and early detection in plain language
- ✓ Ensure information about risk, behaviour change and early detection is culturally appropriate and accessible
- ✓ Address misconceptions or misinformation about cancer risk, behaviour change and early detection
- ✓ Distinguish between increased risk of cancer and a cancer diagnosis and avoid language that catastrophises
- ✓ Supportive care and communication actions related to cancer risk and early detection for [population group] include:
 - list supportive care and communication actions related to cancer risk and early detection specific to this population group; use a new row for each action



Communication and supportive care checklist for people who may have familial / genetic risk factors

<Optional: only include this section if hereditary/genetic risk factors are listed in 1.1.2>

For people who may be at risk due to inherited factors

- ✓ Allow adequate time for discussions about genetic risk and testing for cancer
- ✓ Discuss the person's right to not know genetic information and respect their preference
- ✓ Use plain-language and offer a written summary of options for the person to take away
- ✓ Refer people at familial or genetic risk for genetic counselling to ensure pre- and post-test support
- ✓ Explore the meaning of genetic risk within the patient's cultural and belief framework

For people identified as being at increased risk due to inherited factors

- ✓ Communicate genetic risk clearly and compassionately, without catastrophising
- ✓ Explain the implications for biological relatives in sensitive, non-coercive language
- ✓ Empower people to make informed decisions about testing and family notification at their own pace
 - do not contact family members directly without the person's consent
- ✓ Assess emotional response to identification of familial risk and the impact on family dynamics and relationships, including for relatives who may be obligated to share genetic information
- ✓ Support the person to decide whether, how and to whom to disclose genetic information and offer resources and support for family notification

✓	Supportive care and communication actions related to familial / genetic risk for [population group] include:
–	<i>list communication / supportive care actions about familial / genetic risk specific to this population group</i>

	Find out more
	<u>Resources on supportive care and cancer risk</u>
	<i><Optional> Add resources about risk-related communication and supportive care specific to this population group, including hyperlink</i>
	See also
	Supportive care (Principle) <u><hyperlink to be added></u>
	Communication (Principle) <u><hyperlink to be added></u>
	3.2 Genetic or genomic testing (Pathway step) <u><hyperlink to be added></u>

Step 2: Presentation, initial investigations and referral

Best practice guidance for investigating symptoms, signs and test results associated with cancer, and undertaking timely referral for diagnostic assessment.

2.1 Signs and symptoms



Complete the appropriate box

Cancer-specific OCPs provide lists of signs and symptoms of cancer for each cancer type

Use the boxes below to highlight considerations for presentation of signs and symptoms of cancer specific to this population group.

Population-based factors influencing signs and symptoms of cancer

The following considerations may affect presentation of signs and symptoms of cancer in [population group]:

- list factors that may influence presentation of signs or symptoms of cancer in this population group; use a new bullet point for each factor

Or

There are no known factors influencing presentation of signs and symptoms of cancer specific to [population group].



<Optional> Complete the box

Cancer-specific OCPs highlight the importance of prompt triage by a general practitioner of signs and symptoms of cancer and highlight any symptoms that require urgent assessment. Use the box below to highlight factors that may influence the timeframe for GP consultation that are specific to this population group. Delete the box if there are no specific factors to consider.



Timeframe for general practitioner consultation

Add 1–2 sentences about any factors that may influence timelines that are specific to this population group.

2.2 Assessment by the general practitioner

Following identification of concerning symptoms, or combinations of symptoms, signs and test results, the general practitioner (GP) should undertake appropriate clinical assessment and initial investigations to determine whether urgent or non-urgent referral to a cancer specialist is warranted. Investigation and referral decisions should be tailored to the person's individual health status and social and cultural needs.

Assessment findings, and the decision-making process, should be clearly documented.

All patients should be provided with safety netting advice, including a planned timeframe for follow-up that is agreed with the patient, and the need for patient-initiated follow-up if new symptoms develop, if symptoms persist or worsen, or if the patient has further concerns.



Complete the appropriate box

Population-based considerations for examinations and assessments

The following factors may affect assessment of signs and symptoms of cancer in [population group]:

- *list things the GP should consider as part of examination and assessment for cancer specific to this population; list each factor as a different bullet point*

Or

There are no specific issues or factors a GP should consider as part of assessment of signs and symptoms of cancer in [population group].



<Optional> Complete the box

Cancer-specific OCPs indicate the timeframe within which investigations of signs and symptoms of cancer should be completed following the initial GP consultation. Use the box below to highlight factors that may influence the timeframe for completing investigations specific to this population group. Delete the box if there are no specific factors to consider.



Timeframe for completing investigations

Add 1–2 sentences about any factors that may influence the timeframe for completing investigations that are specific to this population group.

2.3 Initial referral



Complete the appropriate box

When making a referral to investigate signs or symptoms of cancer in [population group], it is important to consider and / or include information about:

- *list population-specific considerations or inclusions in the referral; use a new bullet for each consideration*

Or

There are no specific considerations or inclusions when making a referral for investigation of signs or symptoms of cancer in [population group].

Considerations when making a referral¹⁷

- Referrals do not need to be to a named cancer specialist. People can choose where to present the referral. People should be empowered to make an informed choice about their specialist.
- Referrals should take account of the person's preference and ability to access public or private healthcare and ability to travel.
- Support informed decision making by encouraging the person to ask the cancer specialist about likely costs, waiting periods, and location for further tests and investigations.
- Referrals should contain all relevant clinical information about the patient's condition for investigation, opinion, treatment and management.
- If access is via online referral, a lack of a hard copy should not delay referral.

- Where the referrer is not the person's GP, prompt written communication should be sent to the person's GP about the consultation.
- GPs retain responsibility for the person after referral to a cancer specialist. The GP (or referrer) should be notified if the person does not attend their appointment.



Essential inclusions when referring for suspected or diagnosed cancer

- ✓ Important psychosocial history and relevant medical history
- ✓ Family history, current symptoms, medications and allergies
- ✓ Results of current clinical investigations (e.g. imaging and pathology reports)
- ✓ Results of all prior relevant investigations
- ✓ Notification if an interpreter service is required.



<Optional> Complete the box(es)

Delete boxes if there are no inclusions specific to this population group

Population-specific referral inclusions

Specific inclusions in the referral for investigation of suspected cancer in [population group] include:

- ✓ list information / test results to include in referrals specific to this population group; use a new row for each consideration

Threshold for urgent referral – population-based considerations

The following factors in [population group] should trigger urgent referral for investigation of suspected cancer:

- ✓ list population-specific factors that would influence the requirement for urgent referral



<Optional> Complete the box

Cancer-specific OCPs indicate the timeframe within which a person should see a cancer specialist following investigation of signs and symptoms of cancer by a GP. This includes any factors indicating the need for urgent referral. Use the box below to highlight factors that may influence the timeframe for referral to a cancer specialist specific to this population group. Delete the box if there are no specific factors to consider.



Timeframe for referral to a cancer specialist

Add 1–2 sentences about any factors that may influence the timeframe for referral to a cancer specialist.



Find out more

[Resources on referral for cancer and safety netting](#)

<Optional> Add resources about investigating signs and symptoms / referral specific to this population group, including hyperlink



See also

Person-centred care (Principle) [<hyperlink to be added>](#)

Safety netting: in Safe and quality care (Principle) [<hyperlink to be added>](#)

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations when investigating symptoms of cancer^{18,19}

- People undergoing investigations for cancer should be supported, informed and empowered to make decisions about next steps
- Anxiety, fear and uncertainty can peak when cancer is suspected but not yet confirmed
- Explicit acknowledgement of emotional distress and anxiety is important – conversations should not be deferred
- Support to help people navigate the process of tests and investigations and overcome barriers to attending appointments with a cancer specialist is important
- Optimising health and wellbeing while symptoms are being investigated is important
- Supportive care considerations during investigation of cancer symptoms for [population group] include:
 - list supportive care considerations relevant to investigation of symptoms specific to this population group; use a new row for each consideration



Supportive care checklist

- ✓ Screen for distress and anxiety using a validated tool such as the [NCCN Distress Thermometer](#) and document the results
- ✓ Provide referrals to appropriate sources of information, support and navigation e.g. via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))
 - Specialist support services (via [cancer-specific non-government organisations](#))
- ✓ Consider involvement of a local care coordinator / navigator where available
- ✓ Screen for practical barriers to attending specialist appointments and provide information about:
 - transport
 - out-of-pocket costs
 - leave from work
 - carer responsibilities
- ✓ Encourage physical activity while the person waits for their appointment with the cancer specialist
 - Consider referral to an exercise physiologist if symptoms or deconditioning are present
- ✓ Provide support to manage any physical symptoms. Symptoms that may require specific focus for [population group] include:
 - insert symptoms that need specific focus in this population group

- ✓ Action referrals to allied health or community support services to manage physical symptoms as required
- ✓ Supportive care actions during investigation of cancer symptoms for [population group] include:
 - *list supportive care actions relevant to investigation and referral of symptoms specific to this population group; use a new row for each action*



Communication considerations when investigating symptoms of cancer

- People who understand the reason for referral and what to expect are less anxious and more likely to attend²⁰
- Verbal and written information should be provided to the person including:
 - the name and role of the cancer specialist
 - the reason for referral in plain language
 - expected timeframe for contact
 - what to do if they do not hear from the specialist
 - out-of-pocket costs
 - public vs private options
- Communication considerations during investigation of cancer symptoms for [population group] include:
 - *list communication considerations during investigation and referral for symptoms of cancer specific to this population group; use a new row for each consideration*



Communication checklist

- ✓ Provide the person with verbal and written information about the referral and timeframe (who, why, when)
- ✓ Consider the person's preferred communication format, language and literacy and share this information as part of the specialist referral
- ✓ Organise an interpreter if required
- ✓ Provide clear instructions to contact the referring clinician if the specialist has not been in touch within the expected timeframe
Document this instruction in the patient record
- ✓ Explore options for telehealth consultation with the cancer specialist if appropriate
- ✓ If the referrer is not the GP, send written communication to the person's GP following the consultation
- ✓ Notify the GP if the person does not attend their appointment with the cancer specialist
- ✓ Communication actions during investigation of cancer symptoms for [population group] include:
 - *list communication actions for investigation and referral for symptoms of cancer specific to this population group; use a new row for each action*



Find out more

[Resources for provision of cancer information and support](#)

[Resources for screening for supportive care needs](#)

Resources for practical and social support	
<Optional> Add support and information resources specific to this population group, including <i>hyperlink</i>	
	See also
Supportive care (Principle) <hyperlink to be added> Communication (Principle) <hyperlink to be added> Navigation and care coordination (Principle) <hyperlink to be added>	

Step 3: Diagnosis, staging and treatment planning

Best practice guidance for diagnosing and staging cancer and developing a multidisciplinary treatment plan.

3.1 Specialist diagnostic work-up

Specialist diagnostic work-up starts with a thorough medical history and clinical examination.

3.1.1 Minimum diagnostic tests



Complete the appropriate box

Cancer-specific OCPs list the minimum tests required to diagnose cancer. Use the box below to outline considerations relevant to diagnostic testing for this population group, such as access to tests or interpretation of test results

Population-based diagnostic testing considerations

The following factors may affect or be relevant for diagnostic testing in [population group]:

- list factors influencing for diagnostic testing specific to this population group; use a separate bullet point for each factor

Or

There are no known factors influencing diagnostic testing specific to [population group].



<Optional> Complete the timeframe

Cancer-specific OCPs indicate the timeframe within which diagnostic tests for cancer should be completed following initial presentation or referral to a cancer specialist.

Use the box below to highlight factors that may influence the timeframe for completion of diagnostic tests specific to this population group. Delete the box if there are no specific factors to consider.



Timeframe for completing investigations

Add 1–2 sentences about any factors that may influence the timeframe for completion of diagnostic tests.

3.2 Genetic or genomic testing

Embedding precision medicine diagnostics in routine cancer care can inform diagnostic and treatment decisions. Three distinct but complementary forms of genetic testing can be considered as part of the diagnostic work-up:

- germline testing
- somatic testing
- pharmacogenetic testing

Molecular tumour boards play an important role in the assessment of complex genomic findings.²¹



Key actions for genetic and genomic testing

- ✓ Discuss requirements for genetic or genomic testing at the multidisciplinary team meeting
- ✓ Consider referral to a molecular tumour board (where appropriate and available)
- ✓ Consider the impact of timeframes for genetic or genomic testing on timelines for starting treatment
- ✓ Ensure effective oncology–pathology collaborations to enable timely genetic and genomic testing
- ✓ Order genetic tests as clinically indicated and in accordance with current evidence and guidelines
- ✓ Consider costs of genetic testing
 - Medicare funds some germline tests via the Medicare Benefits Schedule
 - Other tests may be funded through state health systems



<Optional> Complete the box

Use the box below to highlight high-level considerations for genetic or genomic testing for this population group; this may include overarching information about access to testing and / or perceptions of testing that may influence awareness, understanding or consent to undergo testing. You have the option to add information about each type of testing in the relevant sections. Delete this box if there are no considerations specific to this population group.

Population-specific considerations for genetic and genomic testing

The following factors may affect or be relevant for diagnostic testing in [population group]:

- list considerations for genetic or genomic testing specific to this population group; use a new bullet point for each consideration

3.2.1 Germline testing (inherited risk)

Anyone diagnosed with cancer should have a detailed personal and family cancer history taken, covering at least three generations where possible.²² People with a strong family history of cancer may be referred to a familial cancer clinic to determine whether there is an inherited predisposition to cancer. Certain pathological subtypes of cancer, immunohistochemistry or tumour genetic tests may suggest an underlying inherited cancer predisposition.

Finding a germline mutation will have implications for the family. In families with a hereditary cancer predisposition, cascade testing allows for risk-reducing interventions and customised surveillance for those who carry the cancer predisposition genetic variant, thereby facilitating early detection, and resulting in better outcomes.



See also

- For information about genetic risk for [population group] see Section 1.1.3 Genetic testing inherited (germline) cancer risk <hyperlink to be added> <Delete this point if Section 1.1.3 is deleted>
- Multidisciplinary care (Principle) <hyperlink to be added>



Key actions for germline testing

- ✓ Take and document a family cancer history (three generations where possible) at diagnosis
- ✓ Assess whether tumour pathological features suggest a hereditary predisposition
- ✓ Consult [eviQ Referral Guidelines](#) to determine whether referral to a familial cancer service is warranted
- ✓ Ensure pre-test counselling and informed consent before any germline test is ordered
- ✓ Discuss Medicare Benefits Schedule funding and any out-of-pocket costs
- ✓ If a pathogenic variant is identified, facilitate family risk notification and predictive testing for at-risk relatives through a familial cancer service
- ✓ Document all discussions, decisions and referrals in the medical record

3.2.2 Somatic testing (tumour profiling)

Testing for genomic (somatic) variants that originate within the tumour and drive the development and growth of the cancer can enable identification of variant-matched treatments and genomic clinical trials. This can reduce treatment side-effects and improve survival.



<Optional> Complete the box

Cancer-specific OCPs provide information about genomic variants relevant to the cancer type. Use the box below to outline considerations for genomic testing specific to this population group (if not already listed in the introduction to this section).

Delete this box if there are no considerations for genomic testing specific to this population group.

Population-based considerations for somatic testing

Considerations for somatic testing / tumour profiling specific to [population group] include:

- *list considerations for somatic testing specific to this population as bullet points*
- *this may include factors influencing access to tests, collection and storage of tissue and interpretation of test results*



Key actions for somatic testing

- ✓ Determine required somatic tests are indicated
- ✓ Review somatic testing results at the multidisciplinary team meeting to identify matched treatment options or clinical trial eligibility
- ✓ Consider referral to a molecular tumour board if available
- ✓ Discuss results and implications with the patient in plain language, including implications for family members
- ✓ Document all testing decisions, results and treatment implications in the medical record
- ✓ <Optional> List actions related to somatic testing specific to this population group

3.2.3 Pharmacogenetics

Pharmacogenetics can influence the effectiveness of treatment and the risk of adverse effects. Applying pharmacogenetic information to treatment planning can help ensure patients receive the most appropriate drug at the optimal dose from the start of treatment.²³

 Find out more
Resources on genetic testing Resources on cancer genomics
<Optional> Add resources on genomics / somatic testing specific to this population group, including hyperlink

3.3 Staging

Staging is a critical element in treatment planning and should be clearly documented in the patient's medical record.

 **Complete the appropriate box**
Cancer-specific OCPs provide details about tests required to stage each type of cancer.
Use the box below to highlight significant considerations for staging cancer in this population group.

Population-based considerations for cancer staging
Factors that may influence staging for cancer in [population group] include: list considerations for staging cancer specific to this population
Or
There are no specific considerations related to cancer staging in [population group].

 Find out more
Resources on cancer staging
<Optional> Add resources on staging specific to this population group, including hyperlink

3.4 Performance status

Patient performance status is a central factor in cancer care and should be frequently assessed and documented in the patient's medical record.

 **Complete the appropriate box**
Cancer-specific OCPs include tests used to assess performance status appropriate to the cancer type. Use the box below to highlight specific factors that may influence assessment of performance status for this population group

Population-based considerations for assessing performance status
Performance status in [population group] may be influenced by: list factors influencing performance status specific to this population group

Or

There are no specific considerations related to assessing performance status in [population group].



Find out more

[Resources on performance status](#)

<Optional> Add resources on performance status specific to this population group, including hyperlink

3.5 Treatment planning

Treatment planning should be undertaken by a multidisciplinary team. The multidisciplinary team should meet to discuss newly diagnosed patients and recommend a treatment plan. Treatment planning should consider the intent of treatment (curative, control or palliation)²⁴ and treatment goals, taking account of stage of disease and other factors.

Multidisciplinary input should be considered for all patients with cancer. However, not all cases may ultimately necessitate team discussion.²⁵ In such cases, the team should apply agreed treatment protocols. Any people whose cases are not discussed at a multidisciplinary team meeting should at minimum be named on the agenda for noting.

Treatment planning discussions should include consideration of suitable clinical trials or research.

3.5.1 The multidisciplinary team

Multidisciplinary team membership should reflect both clinical and supportive care aspects of care. Pathology and radiology expertise are essential. Additional expertise or specialist services may be required for some patients.



Complete the box

Cancer-specific OCPs list the core members of the multidisciplinary team appropriate to the cancer type. Use the box below to list additional members or types of expertise needed by the multidisciplinary team when discussing treatment plans for this population group.

Population-specific considerations for the multidisciplinary team

When discussing treatment plans for [population group], the multidisciplinary team should include:

- list members and / or expertise specific to this population group



<Optional> Complete the box

Cancer-specific OCPs have the option of providing a timeframe within which multidisciplinary treatment planning should occur following initial review by a cancer specialist. Use the box below to highlight factors that may influence the timeframe for multidisciplinary review and treatment planning specific to this population group. Delete the box if there are no specific factors to consider.



Timeframe for multidisciplinary treatment planning

Add 1–2 sentences about any factors that may influence the timeframe for multidisciplinary treatment planning.



See also

Multidisciplinary care (Principle) [<hyperlink to be added>](#)

Research and clinical trials (Principle) [<hyperlink to be added>](#)

Cancer-specific OCP(s) contain multidisciplinary team expertise required for specific cancer types [<hyperlink to be added>](#)

3.6 Involvement of the GP

A person's GP contributes valuable information to treatment planning. They have knowledge of the person's psychosocial history, comorbidities, medications, access to family and carer support, and locally available support services.

As the health professional with an ongoing relationship with the person and their family, the GP can provide continuity and support while the person comes to terms with their diagnosis and navigates the cancer care system.

If someone does not have a regular GP, they should be encouraged and supported to find one.



<Optional> Complete the box



Key actions for communication with the GP during treatment planning

- ✓ Send timely written communication to the GP about the person's diagnosis and treatment plan
- ✓ Invite the GP to participate in the multidisciplinary meeting (virtual attendance if available)
- ✓ Discuss shared care arrangements and the GP's role during and after treatment
- ✓ List actions about communicating with the GP during treatment planning specific to this population group

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations during diagnosis and treatment planning^{26,27,28}

- Receiving a cancer diagnosis can be a cause of significant psychological distress for the person, their families and carers. Early and proactive identification of supportive care needs is essential.
- A diagnosis of cancer can cause or exacerbate anxiety, depression, adjustment difficulties, treatment phobias, existential concerns, and interpersonal or psychosexual issues.
- Supportive care needs are personal to the individual and influenced by stage of disease, stage of life, gender, relationships, culture and other factors.
- Support to help people navigate the process of diagnosis and treatment planning is important.
- Consideration of family support where the person is a primary caregiver can reduce anxiety and improve acceptability of treatment.
- Prehabilitation before treatment may be appropriate to improve physical and psychological outcomes.²⁹
- For people of childbearing age, discussions about fertility preservation before the start of treatment are essential.³⁰
- Existential and spiritual concerns commonly emerge at diagnosis and during treatment planning.
- For people with advanced cancer, early referral to palliative care can improve outcomes.³¹
- Supportive care considerations during diagnosis and treatment planning for [population group] include:
 - list supportive care considerations at diagnosis specific to this population group; use a new row for each consideration



Supportive care checklist

- ✓ Screen for psychological distress at diagnosis using a validated tool such as the [NCCN Distress Thermometer](#) and document results
 - Activate referral pathways to psychology, social work or counselling as required
- ✓ Provide referrals to appropriate sources of information, support and navigation via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))
 - Specialist support services (via [cancer-specific non-government organisations](#))
- ✓ Provide referrals to appropriate sources of information and support for people considering palliative care e.g. via [Palliative Care Australia](#)
- ✓ Identify whether the person is a primary caregiver and / or has access to family and carer support
- ✓ Identify family and carer distress

✓ Consider involvement of a local care coordinator / navigator where available
✓ Assess and document baseline physical function and performance status
✓ Assess weight status and discuss any implications for treatment planning; refer to a dietitian and/or exercise physiologist as appropriate
✓ Consider completing malnutrition screening using a validated tool (e.g. Malnutrition Screening Tool) and action referral to a dietitian if indicated (note that patients with a high BMI may also be malnourished)
✓ Consider initiating a prehabilitation plan and refer to an exercise physiologist and/or physiotherapist as appropriate
✓ Document smoking or vaping status – Offer brief cessation advice and offer referral to Quitline on 13 7848 if necessary – Prescribe pharmacotherapy if clinically appropriate
✓ Discuss and document fertility implications of recommended treatment where appropriate: – provide contraception advice – discuss and document fertility preservation (including costs) with the person and relevant family or partner before any treatment starts – do not defer this conversation – refer to a fertility specialist if appropriate
✓ Explore and document spiritual / pastoral care needs and make referral if appropriate
✓ Supportive care actions during diagnosis and treatment planning for [population group] include: – <i>list actions for supportive care at diagnosis specific to this population group; use a new row for each consideration</i>
For people diagnosed with advanced cancer
✓ Initiate advance care planning and document preferences
✓ Consider early palliative care referral
✓ Supportive care actions at diagnosis for [population group] with advanced cancer include: – <i>list actions for supportive care for people in this population group with advanced cancer at diagnosis; use a new row for each action</i>



Communication considerations during diagnosis and treatment planning

Communication about the multidisciplinary team

- It is important for people to understand the role of the multidisciplinary team and their ongoing role in treatment and care.
- Consent must be obtained before presenting the person's case at a multidisciplinary meeting.
- The GP is a key member of the team. If the person does not have a regular GP, they should be actively encouraged to establish one.

Communicating a diagnosis of cancer

- Communication of a person's diagnosis, prognosis (if the person wishes to know) and treatment options should be clear, compassionate and at a pace appropriate to the person.

Communicating the treatment plan
• A written treatment plan should include details of the treating team, contact details and instructions on when to call the hospital. • Communication considerations during diagnosis and treatment planning for [population group] include: – <i>list communication considerations during diagnosis and treatment planning specific to this population group; use a new row for each consideration</i>
 Communication checklist
Communication about the multidisciplinary team
✓ Explain the role and benefits of multidisciplinary care and the multidisciplinary team meeting to the patient
✓ Obtain and document written or verbal consent from the person before presenting at the multidisciplinary meeting
✓ Find out whether the person has a regular GP confirmed and if they do not, encourage them to establish one
Communication about the diagnosis
✓ Communicate the diagnosis and prognosis (if the person wants this information) clearly and using plain language
✓ Answer questions clearly and openly and be respectful of the person's preferences for information, cultural beliefs, communication needs and capacity to understand
✓ Provide the Guide to Best Cancer Care for the person's cancer type and population-group
Communication about the treatment plan
✓ Discuss treatment options, including intent, risks, benefits and timeframes
✓ Include discussion of supportive care considerations
✓ Discuss costs and out of pocket expenses
✓ Provide a written treatment care plan that includes: • treating team contact details • information on when and how to contact the hospital • expected treatment timeframe • guidance on managing side effects
✓ Discuss the benefits of continued involvement of primary care during treatment for comorbidity management, health promotion and holistic care
✓ Communication care actions during diagnosis and treatment planning for [population group] include: – <i>list communication actions related to diagnosis and treatment planning specific to this population group; use a new row for each action</i>

 Find out more
Resources for provision of cancer information and support Resources for screening for supportive care needs

Resources for practical and social support Resources on fertility preservation Resources on supportive care and genetic risk Resources on prehabilitation
Guide to Best Cancer Care for [population group] <hyperlink to be added> Plain-language consumer summary of the optimal care pathway for [population group] with cancer, designed to help patients, carers and families understand what care to expect and what questions to ask at each step.
Cancer-specific OCP consumer resources <hyperlink to be added> Each cancer-specific based OCP has a consumer guide to help patients, carers and families understand what care to expect and what questions to ask at each step.
<i><Optional> Add support and information resources relevant to diagnosis and treatment planning specific to this population group, including hyperlink</i>
 See also
Communication about genetic risk (Pathway step 1) <hyperlink to be added> Supportive care (Principle) <hyperlink to be added> Communication (Principle) <hyperlink to be added> Navigation and care coordination (Principle) <hyperlink to be added>

Step 4: Treatment

Best practice guidance for treatment of cancer, including training and experience required of treating clinicians and minimum health service requirements.



<Optional> Complete the box

Cancer-specific OCPs may list population-specific guidelines for individual cancer types



Find out more

For guidance on treatment for [population group] refer to these guidelines:

- *list clinical practice guidelines as a separate bullet and include hyperlinks*

4.1 Treatment decision making



Key actions to enable informed decision-making about treatment

- ✓ Discuss treatment recommendations made by the multidisciplinary team with the patient and their family or carers
- ✓ Obtain and document informed consent for treatment
- ✓ Clearly outline the intent of treatment (curative, control or palliation)
- ✓ Consider and discuss options for relevant clinical trials



See also

Person-centred care (Principle): Informed consent <hyperlink to be added>

Safe and quality care (Principle) <hyperlink to be added>

Research and clinical trials (Principle) <hyperlink to be added>

4.2 Prehabilitation and optimising treatment

Cancer prehabilitation uses a multidisciplinary approach combining exercise, nutrition and psychological strategies to prepare patients for the challenges of cancer treatment such as surgery, systemic therapy and radiation therapy. Team members may include anaesthetists, oncologists, surgeons, haematologists, clinical psychologists, exercise physiologists, physiotherapists and dietitians, among others.



Complete the appropriate box

Cancer-specific OCPs provide details of prehabilitation activity specific to the cancer type. Use the box below to outline prehabilitation considerations specific to this population group – this may include factors such as access, cost or co-morbidities influencing prehabilitation activity

Population-specific considerations for prehabilitation
Factors influencing prehabilitation for [population group] include:
• <i>list details of considerations for prehabilitation specific to this population group</i>
Or
There are no additional factors influencing prehabilitation for [population group].

 Find out more
Resources on prehabilitation
<Optional> Add resources about prehabilitation specific to this population group, including hyperlink

4.3 Treatment overview

4.3.1 Summary

 Complete the box
Cancer-specific OCPs provide a summary of treatments used for individual cancer types / sub-types and stage of disease. The list of treatments used in cancer-specific OCPs is provided below for reference. DO NOT CHANGE THIS LIST. Use the box below the list of treatments to outline overarching considerations for treatment in this population group – for example related to access to treatment, the evidence base in this population group, or cultural considerations. Use this section for high-level considerations only. Use section 4.4 to detail considerations for specific types of treatment.
Treatment modalities
Treatment for cancer involves a combination of: • Surgery • Radiation therapy • Systemic therapy: – Chemotherapy – <u>Targeted therapy</u> – <u>Hormonal therapy</u> – <u>Immunotherapy</u> and <u>precision medicine</u> – <u>Biological and cellular therapy</u> • Transplant • Other – <u>Nuclear medicine</u> – <u>Interventional radiology</u> – <u>Active surveillance</u> – <u>Watch and wait</u> – <u>Complementary therapies</u>

For people diagnosed with de novo advanced (metastatic) [cancer type] treatment also involves:

- palliative care and supportive care.

Clinical trials

Suitable clinical trials should be considered enabling access to innovative therapies or treatment combinations, noting that this may require referral to a different centre. Telehealth can be used to support access.

Overarching treatment considerations for [population group] include:

- *List considerations specific to this population group*



<Optional> Complete the box

Cancer-specific OCPs provide a summary of key timeframes within which treatment for cancer should begin – overall and for specific treatment modalities. Use the box below to highlight factors that may influence the timeframe for starting treatment specific to this population group. Use sub-headings to indicate considerations related to a specific treatment modality if required. Delete the box if there are no specific factors to consider.



Timeframe for treatment

Add 1–2 sentences about any factors that may influence the timeframe for starting treatment .

4.4 Treatment options



Complete the sections below to outline considerations for treatment specific to this population group

- DO NOT CHANGE THE MAIN TREATMENT HEADINGS
- If there are no population-specific considerations for an individual treatment modality, delete the subsection but do not change the heading numbers of remaining sections

4.4.1 Surgery



<Optional> Complete the box

Population-specific considerations for surgery

Factors to consider in relation to surgery for [population group] include:

- *list considerations for surgery specific to this population group*

Population-specific considerations for perioperative treatment

Factors to consider in relation to perioperative treatment for [population group] include:

- *list considerations for perioperative treatment specific to this population group*



Find out more

[See resources on surgery](#)

<Optional> Add resources about surgery specific to this population group, including hyperlink

4.4.2 Radiation therapy

Radiation therapy includes a range of technologies that use radioactive material to treat cancer. Radiation therapy may be used before (neoadjuvant treatment), during (perioperative treatment) or after (adjuvant treatment) definitive treatment for cancer (usually surgery). For some cancer types, radiation therapy may be the definitive treatment offered.



<Optional> Complete the box

Population-specific considerations for radiation therapy

Factors to consider in relation to radiation therapy for [population group] include:

- *list considerations for radiation therapy specific to this population group*



Find out more

[See resources on radiation therapy.](#)

<Optional> Add resources about radiation therapy specific to this population group, including hyperlink

4.4.3 Systemic therapy

Systemic therapy includes a range of different treatment types that may be used before (neoadjuvant treatment), during (perioperative treatment) or after (adjuvant treatment) definitive treatment for cancer (usually surgery).



<Optional> Complete the box

Use the box below to add overarching considerations for systemic therapy in this population group. Or detail considerations specific to a type of systemic therapy in the subsections that follow.

Population-specific considerations for chemotherapy

Overarching factors to consider in relation to systemic therapy for [population group] include:

- *list considerations for chemotherapy specific to this population group*

Chemotherapy



<Optional> Complete the box

Population-specific considerations for chemotherapy

Factors to consider in relation to chemotherapy for [population group] include:

- *list considerations for chemotherapy specific to this population group*

Targeted therapy



<Optional> Complete the box

Population-specific considerations for targeted therapy

Factors to consider in relation to targeted therapy for [population group] include:

- *list considerations for targeted therapy specific to this population group*

Hormonal therapy



<Optional> Complete the box

Population-specific considerations for hormonal therapy

Factors to consider in relation to hormonal therapy for [population group] include:

- *list considerations for hormonal therapy specific to this population group*

Immunotherapy and personalised medicine



<Optional> Complete the box

Population-specific considerations for Immunotherapy and personalised medicine

Factors to consider in relation to immunotherapy and personalised medicine for [population group] include:

- *list considerations for immunotherapy and personalised medicine specific to this population group*

Biological and cellular therapies



<Optional> Complete the box

Population-specific considerations for biological and cellular therapies

Factors to consider in relation to biological and cellular therapies for [population group] include:

- *list considerations for biological and cellular therapies specific to this population group*



Find out more

[See resources on systemic therapies](#)

<Optional> Add resources on systemic therapies specific to this population group, including hyperlink

4.4.4 Palliative care

It is important to consider palliative from the point of diagnosis of advanced (metastatic) cancer. For people with advanced cancer, early integration of palliative care improves symptom management and quality of life, and in some cases is associated with survival benefits.^{32,33,34}

It is essential for services to maintain clear pathways for early referral to palliative care and pain management, particularly if the multidisciplinary team does not include core membership of palliative care.



<Optional> Complete the box

Cancer-specific OCPs include information about palliative care relevant to the cancer type. Use the box below to provide a brief statement about considerations for palliative care in this population group. Detailed information about palliative care is provided in Step 6 – cross reference to this section for detail



Key actions for introduction of palliative care

- ✓ **Initiate early referral** to palliative care services based on need and potential for benefit, not prognosis.
- ✓ **Emphasise the value of palliative care** in improving symptom management and quality of life.
- ✓ **Reassess** palliative care needs at each key clinical decision point.
- ✓ Ensure that **carers and families** receive information, support and guidance about their role in palliative care.³⁵
- ✓ Support the person and their family or carers to initiate **advance care planning**.

Population-specific considerations for palliative care

Factors to consider in relation to palliative care in [population group] include:

- *list considerations for palliative care specific to this population group*



Find out more

[Resources on palliative care](#)

<Optional> Add resources about palliative care specific to this population group, including hyperlink



See also

Step 6: Managing recurrent, residual or metastatic disease (Pathway step) <hyperlink to be added>

Step 7: End of life care (Pathway step) <hyperlink to be added>

4.5 Other treatment options

4.5.1 Nuclear medicine



<Optional> Complete the box

Population-specific considerations for nuclear medicine

Factors to consider in relation to nuclear medicine for [population group] include:

- *list considerations for nuclear medicine specific to this population group*



Find out more

[Resources on nuclear medicine](#)

<Optional> Add resources on nuclear medicine specific to this population group, including hyperlink

4.5.2 Interventional radiology



<Optional> Complete the box

Population-specific considerations for interventional radiology

Factors to consider in relation to interventional radiology for [population group] include:

- list considerations for interventional radiology specific to this population group



Find out more

[Resources on interventional radiology](#)

<Optional> Add resources about interventional radiology specific to this population group, including hyperlink

4.5.3 Watch and wait



<Optional> Complete the box

Population-specific considerations for watch and wait approaches

Factors to consider when considering a watch and wait approach in [population group] include:

- list considerations for a watch and wait approach in this population group

4.5.4 Active surveillance



<Optional> Complete the box

Population-specific considerations for active surveillance

Factors to consider when considering active surveillance in [population group] include:

- list considerations for active surveillance in this population group



Find out more

<Optional> Add resources about watch and wait or active surveillance specific to this cancer, including hyperlink

Complementary therapies may be used alongside conventional cancer treatments to support quality of life, wellbeing and symptom management.³⁶ While some complementary therapies are supported by evidence, others are not.³⁷ Integrative oncology combines evidence-based complementary therapies with conventional cancer care to address the physical, psychological and social impacts of cancer.³⁸³⁹

A person's decision to use complementary therapies should not affect their access to or the quality of care they receive.⁴⁰



<Optional> Complete the box



Key actions in relation to complementary therapies⁴¹

- ✓ **Proactively identify and document** a person's current or planned use of complementary therapies.
- ✓ **Assess each therapy** for safety, efficacy and to identify any potential toxicity or drug interaction.⁴²
- ✓ **Communicate openly** and without judgement when exploring the person's reasons for wanting to use complementary therapies.
- ✓ Consider both the **evidence of benefit and the costs** of therapies during discussions.
- ✓ Where appropriate, **discuss evidence-based integrative oncology approaches** that may support symptom management, wellbeing and quality of life, such as physical activity, mindfulness-based interventions, yoga, acupuncture or oncology massage.⁴³
- ✓ If a person expresses interest in using complementary therapies, consider **referral to a multidisciplinary team member with relevant expertise** (e.g. a clinical pharmacist, dietitian or psychologist) to help them reach an informed decision.

Population-specific considerations for complementary therapies

Factors to consider in relation to use of complementary therapies in [population group] include:

- *list considerations for use of complementary therapies specific to this population group*



Find out more

[See resources on complementary therapies](#)

<Optional> Add resources about complementary therapies specific to this population group, including hyperlink

4.6 Involvement of the general practitioner in treatment for cancer

Early and sustained involvement of the person's general practitioner (GP) during active treatment improves outcomes post-treatment.⁴⁴ The role of the GP during treatment may include support for symptom management, management of co-morbidities, emotional and psychosocial support and referral to local services.

The GP can act as a consistent support for people and their families between specialist appointments.

A person's GP should receive regular, timely communication throughout treatment, not only at the end of treatment.



<Optional> Complete the box

Consider adding a brief statement about communicating with the GP about cancer treatment for this population group. This may include involvement of other primary care providers involved in the person's care



Key actions for communicating with the GP during treatment

- ✓ Two-way communication should cover:
 - treatment plan, treatment intent (curative intent, control, palliation) and potential side effects

- changes to treatment or medications
- supportive and palliative care requirements
- the patient's prognosis and their understanding of it
- clinical trial enrolment
- the presence of an advance care directive or substitute decision-maker
- multidisciplinary team meeting recommendations.

✓ Consider use of a Chronic Condition Management Plan (GPCCMP) (can be billed as an MBS item)

Population-specific considerations for communicating with the GP about treatment for cancer

Considerations for communicating with the GP about treatment for [population group] include:

- *list considerations for communicating with the GP specific to this population group*



Find out more

[Information about developing a GP Chronic Condition Management Plan](#)

4.7 Health professional training and experience for safe delivery of cancer treatment



<Optional> Complete the box

Cancer-specific OCPs include detailed information about the workforce training and experience needed for health professionals involved in treatment for specific cancer types. Use this box to highlight training, credentials or experience needed by health professionals involved in treatment for this population group.

Population-specific training and experience required by health professionals involved in treatment for cancer

Health professionals involved in providing treatment and care for [population group] with cancer should have training and experience in:

- *list training and experience considerations for this population group*

4.10 Health service characteristics: considerations for this population group



<Optional> Complete the box

Cancer-specific OCPs include detailed information about the health service characteristics needed to deliver safe and high-quality treatment for individual cancer types. Use this box to highlight health service features needed to deliver appropriate treatment and care to this population group.

Population-specific health service characteristics for delivery of safe and effective treatment for cancer

Health services delivering treatment and care for [population group] with cancer should include:

- *list health service characteristics specific to this population group*

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations during treatment^{45,46,47}

- It is important to screen for supportive care needs at the start of treatment and when the intent of treatment changes. A person's needs can shift significantly during active treatment.
- Family members and carers may also have supportive care needs.
- People may have needs across all five domains of supportive care during treatment.
- Support to help people navigate through their treatment pathway is important.
- Common supportive care needs for people with cancer are listed in the [Supportive care principle](#) <hyperlink to be added>
- Supportive care needs during treatment specific to [population group] include:
 - list supportive care needs during treatment specific this population group; use a new bullet for each need

Rehabilitation^{48,49}

- People undergoing cancer treatment may experience a range of symptoms and side effects that affect their mobility and functional status which in turn influences activities of daily living and capacity for work.
- Referrals to services such as occupational therapy, speech pathology, dietetics, physiotherapy, exercise physiology, social work and psychology are important to assist with rehabilitation.
- Exercise is a safe and effective intervention throughout active cancer treatment and should be embedded as standard practice, not added as a supplement.^{50,51,52}
- Exercise must be individually prescribed and delivered by an accredited exercise physiologist or physiotherapist with cancer care experience, tailored to the patient's current capacity, with a goal of transitioning to ongoing self-managed exercise.
- Rehabilitation considerations for [population group] include:
 - list rehabilitation considerations specific to this population group; use a new bullet for each consideration

Social considerations

- People who are living away from home for treatment are at particular risk of social isolation.
- Financial hardship is common during treatment due to loss of income, reduced work capacity and out-of-pocket treatment costs.
- People may need advice and support for legal considerations such as wills, care of dependents, and insurance, superannuation or social security claims on the basis of illness or disability.
- Social considerations during treatment specific to [population group] include:
 - list social considerations during treatment specific to this population group; use a new bullet for each consideration



Supportive care checklist

- ✓ Screen for and document supportive care needs at the start of treatment and when treatment or treatment intent changes
- ✓ Provide referrals to appropriate sources of information, support and navigation via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))
 - Specialist support services (via [cancer-specific non-government organisations](#))
- ✓ Provide referrals to appropriate sources of information and support for people considering or receiving palliative care e.g. via [Palliative Care Australia](#)
- ✓ Screen for psychological distress using a validated tool such as the [NCCN Distress Thermometer](#) and document results
 - Activate referral pathways to psychology, social work or counselling as required
- ✓ Assess family and carer needs and offer referrals as needed
- ✓ Assess rehabilitation needs and refer for relevant allied health support as required; consider:
 - exercise prescription / referral to exercise physiologist or physiotherapist with cancer experience
 - nutritional status with referral to dietitian maintained or initiated if indicated
 - assessment of cancer-related fatigue assessed and provide information and self-management strategies
- ✓ Assess financial and practical needs and provide information and referrals for support as required; consider:
 - financial impacts of treatment
 - legal requirements
 - isolation risks
- ✓ Discuss and / or review and update [advance care planning](#)
- ✓ Supportive care actions during treatment for [population group] include:
 - *list supportive care actions during treatment specific to this population group; use a new bullet for each consideration*



Communication considerations during treatment

- Clear communication is essential at every stage of active treatment. Discussions should cover:
 - treatment intent and expected outcomes
 - possible side effects and how to manage them
 - how to access care and emergency contacts
 - self-management strategies between active treatments
 - the role of supportive and palliative care as relevant
- Informed decision making can be actively supported by:
 - providing written copies of information resources

- encouraging patients to bring a support person to appointments
- using question prompt lists
- audio-recording consultations where helpful.

- Communication considerations during treatment for [population group] include:
 - *list communication considerations during treatment specific to this population group; use a new bullet for each consideration*



Communication checklist

- ✓ Discuss treatment intent, options and expected outcomes with the person and carer and provide written information/resources and referrals for professional and community support
- ✓ Provide clear information on side effect management, self-management strategies and who to contact in an emergency
- ✓ Support informed decision-making and encourage question prompt lists, audio recordings of consultations, and a support person at appointments
- ✓ Discuss and document advance care planning
- ✓ Maintain structured, timely two-way communication with the GP throughout treatment
- ✓ Communication actions during treatment for [population group] include:
 - *list communication actions during treatment specific to this population group; use a new row for each action*



Find out more

[Resources for provision of cancer information and support](#)

[Resources for screening for supportive care needs](#)

[Resources for practical and social support](#)

[Resources on fertility preservation](#)

[Resources on prehabilitation](#)

<Optional> Add information and support resources for people in this population group undergoing treatment for cancer, including hyperlink



See also

Supportive care (Principle) [<hyperlink to be added>](#)

Communication (Principle) [<hyperlink to be added>](#)

Navigation and care coordination (Principle) [<hyperlink to be added>](#)

Step 5: Care after initial treatment and recovery

Best practice guidance for supporting health and wellbeing following completion of active treatment for cancer, including follow-up care.

This step focuses on post-treatment care and recovery after treatment with curative intent. See Step 6 for guidance on supporting health and wellbeing for people living with advanced (metastatic) cancer.

5.1 Transitioning to post-treatment care

The transition to post-treatment care is critical to long-term health. Some people need ongoing, hospital-based care. For others, a shared follow-up care arrangement with a general practitioner (GP) or nurse may be appropriate. This will vary depending on the type and stage of cancer and needs to be planned.

Models of follow-up care following completion of active treatment:^{53,54}

- Specialist-led care (hospital-based)
- Shared care (specialist and GP)
- GP-led or primary care-led
- Nurse-led models⁵⁵

Follow-up care may be supported by telehealth.

The appropriate model of follow-up care depends on cancer type, stage, predicted risk and the patient's individual clinical and psychosocial needs. Promoting self-management of emotional, psychosocial, physical, and functional impacts of cancer and its treatment is integral to follow-up care.



Key actions at the end of active treatment

- ✓ At the end of initial treatment, a designated member of the multidisciplinary team should meet with the patient to:
 - complete a needs assessment using a validated tool such as the [NCCN Distress Thermometer](#) and document results
 - activate referral pathways to psychology, social work or counselling as required
 - prepare and deliver a [treatment summary](#)
 - develop a [survivorship care plan](#) in partnership with the patient
 - agree and clearly document the follow-up model of care with the lead clinician, GP and relevant multidisciplinary team members
 - ensure the patient understands the rapid re-entry pathway and the specific symptoms of recurrence to report.
- ✓ Provide copies of the treatment summary and [survivorship care plan](#) to the patient, their carer (if appropriate) and the person's GP.
- ✓ Consider opportunities to participate in clinical trials or research related to survivorship care and long-term health and wellbeing.



See also

Research and clinical trials (Principle) [<hyperlink to be added>](#)

5.2 Supporting long-term health and wellbeing



Complete the box(es)

Cancer-specific OCPs will list common physical, psychological and social considerations relevant to the cancer type

Provide a brief list (bullet points) below of issues related to long-term health and wellbeing that are unique to or require heightened focus for this population group (e.g. infertility for adolescents and young adults)

Physical considerations

Common long-term physical effects of treatment for cancer may include:

- cardiovascular toxicity
- endocrine effects
- neuropathy
- sexual dysfunction
- infertility
- cognitive changes
- fatigue
- impaired cardiac or pulmonary function
- risk of second malignancies.

Long-term physical effects of cancer that require particular focus in **<population group>** include:

- *list physical effects that require specific consideration in this population group as bullet points*

Psychological considerations

- Fear of cancer recurrence (FCR) is the most commonly reported unmet need for people following completion of active treatment. People experience high levels of uncertainty about the future and fear of relapse.

- Other common psychological considerations for cancer include:

- anxiety
- depression
- adjustment difficulties
- sleep disturbance.

- Changes to sexual function, body image, fertility and intimate relationships are common survivorship concerns and are consistently under-assessed and underdiscussed.^{56,57}

- Carers also frequently experience distress, grief, and adjustment challenges during and after the person's treatment.⁵⁸

Long-term psychological impacts of cancer that require particular focus in **<population group>** include:

- *list psychological effects that require specific consideration in this population group as bullet points*

Social considerations
- Financial hardship is common and often persists long after treatment.^{59,60} - People may need support with insurance claims, superannuation, social security entitlements, legal matters such as wills or care of dependent children, and advance care planning.
Long-term social impacts of cancer that require particular focus in <population group> include: <List effects as bullet points>

5.3 Plans following completion of treatment

5.3.1 Treatment summary

 <Optional> Complete the box	
 Key inclusions in the treatment summary	
✓	Diagnostic tests performed and results
✓	Diagnosis including stage, prognostic or severity score
✓	Tumour characteristics
✓	Treatment received (types and dates)
✓	Current toxicities (severity, management and expected outcomes)
✓	Interventions and treatment plans from other health providers
✓	Potential long-term and late effects of treatment
✓	Supportive care services provided
✓	Follow-up schedule
✓	Contact information for key healthcare providers.
✓	List any inclusions in the treatment summary specific to this population group

5.3.2 Follow-up care plan

 <Optional> Complete the box	
 Key inclusions in the follow-up care plan	
Follow-up appointments	- Agreed frequency and duration of follow-up appointments - Who is responsible for follow-up - Triggers for escalation back to specialist care - Process and contact details for rapid re-entry to specialist services for suspected recurrence
Cancer surveillance	- Specific tests required and their timing - Surveillance for secondary and metachronous cancers - Screening for medical and psychosocial late effects

Signs and symptoms	✓ Symptoms that may indicate recurrence and require prompt review
Late effects	✓ Possible treatment-specific late and long-term effects ✓ Plan monitoring, screening and management for relevant effects
<Add inclusion>	✓ List inclusions in the follow-up care plan specific to this population group

5.3.3 Survivorship care plan / wellness plan

A comprehensive survivorship care plan extends beyond medical follow-up to support overall wellness. It should be grounded in an understanding of the person's cancer and treatments and should support self-management as well as providing information and referrals to relevant sources of support as appropriate. Uploading of the plan to My Health Record can support implementation, coordination and ongoing monitoring.

 <Optional> Complete the box	
 Key inclusions in a survivorship / wellness plan	
Physical activity	✓ Recommendations for exercise tailored to the patient's capacity, treatment history and goals ✓ Referral to physiotherapist or exercise physiologist where indicated
Nutrition	✓ Recommendations for diet and nutrition ✓ Referral to dietitian where nutritional needs are complex or treatment has affected eating
Wellbeing and risk reduction	✓ Recommendations for ongoing wellbeing and to reduce risk of recurrence: • maintaining a healthy body weight • smoking or vaping cessation • reduce or avoid alcohol • sun smart behaviours
Mental health and wellbeing	✓ Screen for fear of cancer recurrence, anxiety, depression and cognitive changes at follow-up appointments ✓ Consider developing a mental health plan if required ✓ Refer to psychology, social work or peer support as indicated
Practical and financial wellbeing	✓ Identify financial hardship, return to work/study concerns and carer impacts early ✓ Connect patients to financial counselling, social work and relevant support services
Fertility counselling	✓ Recommendations for ongoing contraception needs (noting that some treatments require contraception for a defined period post-treatment) ✓ Referral to a fertility specialist for people of reproductive age who wish to conceive ✓ Assess for signs of premature ovarian insufficiency or hypogonadism and refer to an endocrinologist or gynaecologist as appropriate
<Add inclusion>	✓ List inclusions in the survivorship care plan specific to this population group



Find out more

[See resources for provision of cancer information and support](#)

[See resources for screening for supportive care needs](#)

[See resources on survivorship care](#)

<Optional> Add resources about survivorship care specific to this population group, including hyperlink

5.4 Involvement of the GP

Care following completion of active treatment for cancer should be integrated.⁶¹ The person's general practitioner (GP) has an important role in ongoing follow-up care including:⁶²

- management of long-term and late effects of treatment
- management of comorbidities
- connecting the person to local community services and programs
- providing ongoing wellbeing and self-management advice
- screening for cancer and non-cancerous conditions.

The GP's role in symptom management, comorbidity care, mental health and local referral should be actively supported and clearly communicated.



<Optional> Complete the box



Key actions for communication with the GP during follow-up

- ✓ Two-way communication should cover:
 - the patient's progress
 - the follow-up care plan
 - potential late effects
 - supportive and palliative care requirements
 - any shared care arrangements
 - clarification of various roles in patient care
 - a process for rapid re-entry to medical services for patients with suspected recurrence or if there are other concerns.
- ✓ Consider use of a Chronic Condition Management Plan (GPCCMP) (can be billed as an MBS item)
- ✓ Add actions about communicating with the GP about survivorship care specific to this population group



Find out more

[Information about developing a GP Chronic Condition Management Plan](#)

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations after completion of active treatment

- Fear of cancer recurrence (FCR) is the most commonly reported unmet need among cancer survivors. It should be routinely assessed at follow-up appointments, not assumed to diminish over time.^{63,64}
- Anxiety, depression, cognitive changes, sleep disturbance and adjustment difficulties are common in survivorship.
- Carers frequently experience their own distress, grief, and adjustment challenges during and after the patient's treatment.⁶⁵
- People can face unique challenges with financial hardship, return to work or study, ongoing care costs and legal matters.
- Changes to sexual function, body image, fertility and intimate relationships are common survivorship concerns and are consistently under-assessed and underdiscussed.^{66,67}
- People need information, tools and support to manage their health after initial treatment, including how to monitor for symptoms of recurrence, when to seek urgent review, how to access services, and how to maintain the wellness behaviours outlined in their survivorship care plan.
- Supportive care considerations after completion of active treatment for [population group] include:
 - *list supportive care considerations after active treatment specific to this population group; use a new row for each consideration*



Supportive care checklist

- ✓ Re-screen for supportive care needs using a validated tool such as the [NCCN Distress Thermometer](#) or [CaSun](#) and document the results
- ✓ Assess for fear of cancer recurrence assessed and refer to relevant services
- ✓ Refer the person to appropriate sources or information and support for financial, legal and employment concerns
- ✓ Refer the person to appropriate sources of information and support for concerns related to intimacy, sexuality and body image
- ✓ Provide the person with appropriate information and support for self-management, such as [MyCarePlan](#).
- ✓ Provide referrals to appropriate sources of information, support and navigation via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))
 - Specialist support services (via [cancer-specific non-government organisations](#))
- ✓ Supportive care actions after completion of active treatment for [population group] include:

- *list supportive care actions following completion of active treatment specific to this population group; use a new row for each action*



Communication considerations following completion of active treatment

- A person should be involved in discussions about their survivorship care plan
- It is important for a person to understand the role of the health professionals involved in their follow up care
- It is important a person knows who to contact if they have any concerns about signs and symptoms or recurrent disease
- Communication considerations after completion of active treatment for [population group] include:
 - *list communication considerations following completion of active treatment specific to this population group; use a new row for each consideration*



Communication checklist

- ✓ Develop a treatment summary and follow-up care plan in consultation with the person and the multidisciplinary team. Discuss the roles of the health professionals involved in follow-up care with the person
- ✓ Provide the person with information on the signs and symptoms of recurrent disease, secondary prevention and healthy living
- ✓ Provide the person with contact details for the care team, and instructions on when to get in touch
- ✓ Communication actions after completion of active treatment for [population group] include:
 - *list communication actions after completion of active treatment specific to this population group; use a new row for each action*



Find out more

[Resources for provision of cancer information and support](#)

[Resources for screening for supportive care needs](#)

[Resources for practical and social support](#)

[Resources on survivorship care](#)

<Optional> *Add resources about survivorship care specific to this population group, including hyperlink*



See also

Supportive care (Principle) [<hyperlink to be added>](#)

Communication (Principle) [<hyperlink to be added>](#)

Navigation and care coordination (Principle) [<hyperlink to be added>](#)

Step 6: Managing residual, recurrent or metastatic disease

Best practice guidance for management of residual, recurrent or metastatic disease. Includes guidance about palliative care and advance care planning and about supporting health and wellbeing for people living with advanced (metastatic) cancer.

6.1 Detecting residual, recurrent or metastatic disease

Residual, recurrent and metastatic cancer are clinically distinct states. They may be detected through:

- **routine surveillance:** detected on scheduled imaging, blood tests or clinical review during the post-treatment follow-up period
- **incidental finding:** detected during investigation for an unrelated condition.
- **patient-reported symptoms:** which may be new or worsening, at or between scheduled appointments.

Signs and symptoms will depend on the location of residual, recurrent or metastatic disease.



Complete the appropriate box

Cancer-specific OCPs list signs and symptoms of residual, recurrent and metastatic cancer.

Use the box below to highlight key factors that may influence detection of residual, recurrent or metastatic cancer specific to this population group, such as atypical presentation, communication barriers or less intensive surveillance

Population-specific factors influencing detection of residual, recurrent or metastatic cancer

Detection of residual, recurrent or metastatic disease in [population group] may be influenced by factors such as:

- *list key factors influencing detection of residual, recurrent or metastatic cancer specific to this population group*

Or

Detection of signs and symptoms of recurrent, residual or metastatic cancer in [population group] are similar to those described for initial presentation. See Step 2 for details.

6.2 Investigating residual, recurrent or metastatic disease

Action should be taken immediately on of suspected residual, recurrent or metastatic disease.



<Optional> Complete the box



Key actions to investigate residual, recurrent or metastatic disease

- ✓ Arrange appropriate staging investigations to confirm and characterise the extent of disease
- ✓ Re-biopsy where clinically indicated
- ✓ Tumour biology can change, and receptor or molecular status should be re-evaluated to inform treatment planning

✓ Refer or re-refer to the specialist MDT without delay
✓ Assess and document performance status and comorbidities to guide treatment planning
✓ Initiate or reassess supportive care and psychosocial needs
✓ List actions for investigating residual, recurrent or metastatic disease specific to this population group

6.3 Managing residual, recurrent or metastatic disease

6.3.1 Advanced disease states and treatment intent

Understanding and communicating the disease state and treatment intent is essential for treatment planning. Treatment intent may change from symptom control to palliation as the disease evolves. Treatment intent and goals of care should be discussed with the person and their family / carer at each key decision point and documented in the medical record.

6.3.2 Treatment planning and the multidisciplinary team

All patients with confirmed or suspected residual, recurrent or metastatic disease must be reviewed by a multidisciplinary team. The complexity of advanced disease requires input from multiple specialties to ensure all treatment options are considered. The multidisciplinary team for advanced or metastatic cancer may include additional / different members from the team involved in developing the initial treatment plan.



See also

Multidisciplinary care (Principle) [<hyperlink to be added>](#)



Complete the box below

Cancer-specific OCPs list core multidisciplinary team membership appropriate to planning care for people with residual, recurrent or metastatic cancer

Use the box below to identify members or expertise required when planning the care of people in this population group

Population-specific considerations for membership of the multidisciplinary team for people with residual, recurrent or metastatic cancer

Multidisciplinary team members and expertise required when discussing residual, recurrent or metastatic cancer in [population group] should include:

- *list additional members or expertise required for this population group*



<Optional> Complete the box



Key inclusions for multidisciplinary team discussion about residual, recurrent or metastatic disease

- ✓ Extent and location of disease
- ✓ Prior treatment history, tolerances and toxicities
- ✓ Current performance status and comorbidities
- ✓ All available treatment options, including clinical trials
- ✓ Locoregional options (e.g. surgery, radiotherapy, ablation) where oligometastatic disease is present
- ✓ Goals of care and patient preferences
- ✓ Palliative care needs
- ✓ Advance care planning status
- ✓ List inclusions for multidisciplinary team discussion about residual, recurrent or metastatic cancer specific to this population group



<Optional> Complete the box



Treatment planning checklist for residual, recurrent or metastatic disease

- ✓ Patient formally discussed at multidisciplinary team meeting with appropriate specialties present
- ✓ Treatment intent agreed and documented
- ✓ All treatment options considered, including locoregional therapies
- ✓ Clinical trial options considered
- ✓ Goals of care discussed with patient and carer and documented
- ✓ Treatment plan communicated to patient, carer and GP in writing
- ✓ List inclusions for the treatment planning checklist relevant to residual, recurrent or metastatic cancer specific to this population group



See also

Research and clinical trials (Principle) [<hyperlink to be added>](#)



For people with advanced cancer, early integration of palliative care improves symptom management and quality of life, and in some cases is associated with survival benefits.^{68,69,70}

6.4 Treatment for residual, recurrent or metastatic disease

Treatment options for residual, recurrent or metastatic cancer depend on:

- the location and extent of disease
- whether disease is locoregional or distant
- prior treatment received and cumulative toxicities
- tumour biology at the time of advanced disease (which may differ from initial diagnosis)
- the person's performance status, comorbidities and preferences.



<Optional> Complete the box

Cancer-specific OCPs list treatment modalities used for residual, recurrent or metastatic cancer. Consider using the box below to outline key considerations for treatment of residual, recurrent or metastatic cancer specific to this population group. Use subheadings as necessary to delineate differences in treatment options for residual / recurrent / metastatic disease; cross reference step 4 content as required.

Treatment considerations for residual, recurrent or metastatic cancer in [population group] may include:

- *list population-specific treatment considerations*

6.5 Supporting health and wellbeing for people with advanced (metastatic) cancer

Improvements in cancer surveillance, diagnostics, and treatment mean that people with advanced (metastatic) cancer are living for long periods with disease control. A focus on survivorship care is as essential for people with advanced cancer as it is for people with earlier stage disease.⁷¹ COSA-endorsed MASCC-ASCO Standards and Practice Recommendations highlight the importance of prioritising and providing quality and consistent survivorship care for people affected by advanced (metastatic) cancer from the point of diagnosis.⁷²



<Optional> Complete the boxes

Cancer-specific OCPs will list specific considerations around supporting health and wellbeing in people with metastatic or advanced forms of individual cancer types. Use the boxes below to highlight health and wellbeing considerations specific to this population group when living with metastatic cancer.

Considerations for health and wellbeing highlighted in Step 5 are relevant following a diagnosis of advanced or metastatic disease. However, contributing factors may be different, and the frequency and intensity of needs may vary.⁷³

Physical needs for people with advanced (metastatic) cancer⁷⁴

Common physical needs include:

- managing pain
- maintaining independence
- preserving physical function
- fatigue and lack of energy

• adapting to life roles and daily activities • managing side effects or complications
Physical needs specific to [population group] with advanced (metastatic) cancer include: • <i>list physical needs specific to people in this population group with advanced (metastatic) cancer</i>
Psychological needs for people with advanced (metastatic) cancer⁷⁵
Common psychological needs include: • uncertainty • fear of cancer progression • fear of physical deterioration and pain • depression and anxiety • stress and worry
Psychological needs specific to [population group] with advanced (metastatic) cancer include: • <i>list psychological needs specific to people in this population group with advanced (metastatic) cancer</i>
Social considerations for people with advanced (metastatic) cancer⁷⁶
Common social / financial needs include: • loss of income • extra expenditure • financial planning • assistance with insurance • accessing social services
Social needs specific to [population group] with advanced (metastatic) cancer include: • <i>list social needs specific to people in this population group with advanced (metastatic) cancer</i>
Other population-specific considerations for health and wellbeing for people with advanced (metastatic) cancer
Other survivorship considerations for [population group] living with advanced (metastatic) cancer include: • <Optional>: <i>list specific survivorship care considerations for this population group related to advanced (metastatic) cancer</i>



See also

5.2 Supporting long-term health and wellbeing (Pathway step) [<hyperlink to be added>](#)

6.6 Palliative care

Palliative care is a core component of managing residual, recurrent and metastatic cancer. Palliative care services are provided by a range of health professionals. Involvement of different health professionals should be considered based on individual needs of the person receiving care.⁷⁷

Access to palliative care may be enabled by telehealth.^{78,79}

Referral for palliative care support should be based on a person's need rather than prognosis.⁸⁰



ASCO guideline recommendations for palliative care for people with cancer⁸¹

- ✓ Refer people with advanced solid tumours and haematological malignancies to specialised interdisciplinary palliative care teams early in the course of the disease alongside active treatment
 - this includes people on clinical trials
- ✓ Provide dedicated palliative care services that complement existing and emerging supportive care interventions to address physical, psychosocial and spiritual distress
- ✓ Refer family and carers to palliative care teams for additional support



<Optional> Complete the box below

Cancer-specific OCPs list specific palliative care considerations for individual cancer types

Use the box below to highlight factors influencing palliative care specific to this population group



Factors influencing palliative care

Palliative care considerations for [population group] include:

- list considerations for palliative care specific to this population group



Find out more

[Resources about palliative care](#)

<Optional> Add palliative care resources specific to this population group, including hyperlink



See also

Step 4 Treatment (Pathway step) <hyperlink to be added>

Research and clinical trials (Principle) <hyperlink to be added>

6.7 Advance care planning

Advance care planning is important for all patients with a cancer diagnosis but especially those with advanced disease.

Patients should be encouraged to think and talk about their healthcare values and preferences with family or carers, appoint a substitute decision-maker and consider developing an advance care directive to convey their preferences for future health care in the event they become unable to communicate their wishes.



Find out more

[Resources about advance care planning](#)

<Optional> Add advance care planning resources specific to this population group, including hyperlink



See also

Step 6: End-of-life care (Pathway step) [<hyperlink to be added>](#)

Voluntary assisted dying [<hyperlink to be added>](#)

6.8 Involvement of the general practitioner

The person's general practitioner (GP) plays an important role in symptom management, management of comorbidities, psychosocial support and referral to local and community-based services. For some people, the GP may be the primary provider of palliative care.

A diagnosis of advanced disease, and any change in treatment intent, should be communicated to the GP promptly so that care remains coordinated.

As palliative care is introduced, the GP is well placed to work alongside the palliative care team and other specialists and to support the person and their family with advance care planning. The GP's role in symptom management, comorbidity care, mental health and local referral should be actively supported and clearly communicated.



Key actions for communication with the GP in relation to residual, recurrent or metastatic disease

- ✓ Two-way communication should cover:
 - the treatment plan and intent
 - changes to treatment
 - supportive and palliative care needs
 - prognosis
 - any advance care directive or substitute decision-maker
 - shared care arrangements.
- ✓ Consider use of a Chronic Condition Management Plan (GPCCMP) (can be billed as an MBS item)



Find out more

[Information about developing a GP Chronic Condition Management Plan](#)

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations for people with residual, recurrent or metastatic disease

- A new diagnosis of advanced disease is a significant event for patients and carers. Common unmet needs for people with advanced (metastatic) cancer are listed in section 6.5. [<hyperlink to be added>](#)
- Recurrent, residual or metastatic disease often brings fear of death and dying, existential distress and anticipatory grief.
- Declining mobility and functional status are common with recurrent disease.
- Cognitive changes from treatment or disease progression can affect memory, attention and concentration.
- A substitute decision-maker and an advance care plan / directive can be vital to ensure a person's wishes are addressed at this time.
- Loss of income and extra expenditure on treatment and care can cause financial hardship. People may need financial planning advice or assistance with access to superannuation, insurance or social services.⁸²
- Support with legal matters such as wills, care of dependent children, and terminal-illness or permanent-disability claims may be needed.
- Carers and family members need their own assessment and support, including respite.
- Supportive care considerations for [population group] with residual, recurrent or metastatic disease include:
 - *list supportive care considerations specific to people in this population group with residual, recurrent or metastatic; use a new row for each consideration*



Supportive care checklist

- ✓ Re-screen for supportive care needs using a validated tool such as the [NCCN Distress Thermometer](#) and document the results
- ✓ Assess psychological distress, fear of death and dying, and anticipatory grief, and refer to relevant services
- ✓ Discuss and document substitute decision-maker arrangements and an advance care directive
- ✓ Assess physical symptoms and mobility, and refer to palliative care, physiotherapy and occupational therapy as required
- ✓ Assess carer and family needs and offer support, respite and referral
- ✓ Provide referrals to appropriate sources of information, support and navigation via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))

– Specialist support services (via cancer-specific non-government organisations)
✓ Provide referrals to appropriate sources of information and support for people receiving palliative care e.g. via Palliative Care Australia
✓ Supportive care actions for [population group] with residual, recurrent or metastatic disease include: – <i>list supportive care actions considerations specific to people in this population group with residual, recurrent or metastatic; use a new row for each consideration</i>



Communication considerations for people with residual, recurrent or metastatic disease

- A person may need support to communicate their wishes to loved ones and to navigate interpersonal and relationship strain.
- Communication considerations for [population group] with residual, recurrent or metastatic disease include:
 - *list communication considerations specific to people in this population group with residual, recurrent or metastatic; use a new row for each consideration*



Communication checklist

- ✓ Check the person's understanding of their diagnosis and prognosis and tailor the conversation to what they want to know
- ✓ Hold sensitive, timely conversations about goals of care, advance care planning and the role of palliative care
- ✓ Provide the person with contact details for the care team and instructions on when to get in touch
- ✓ Establish regular, two-way communication with the person's GP, covering the treatment plan, prognosis, supportive and palliative care needs, and shared care arrangements
- Supportive care considerations for [population group] with residual, recurrent or metastatic disease include:
 - *list supportive care considerations specific to people in this population group with residual, recurrent or metastatic; use a new row for each consideration*



Find out more

[Resources about palliative care](#)
[Resources for practical and social support](#)
[Resources about survivorship care](#)

<Optional> Add resources about supportive care or communication for people with residual, recurrent or metastatic disease specific to this population group, including hyperlink



See also

Step 7 End-of-life care (Pathway step) [<hyperlink to be added>](#)
 Supportive care (Principle) [<hyperlink to be added>](#)
 Communication (Principle) [<hyperlink to be added>](#)

Step 7: End-of-life care

Best practice guidance for maintaining quality of life and meeting health and supportive care needs as people with cancer approach the end of life. Includes guidance for supporting family and carers.

7.1 End-of-life care planning

7.1.1 Goals of end-of-life care

End-of-life care focuses on maintaining quality of life, managing symptoms, supporting a person's values and preferences, and meeting the needs of families and carers. Safe and high-quality end-of-life care should be:

- person-centred and include family and carers
- align with values, needs and wishes
- consider cultural, spiritual and psychosocial needs
- not offer unreasonable hope.⁸³

7.1.2 Transitioning to end-of-life care

The first step in providing safe and high-quality end-of-life care is to recognise people who would benefit from such care.⁸⁴

Key clinical triggers indicating end-of-life care is appropriate
• Functional status declining despite optimal management (ECOG performance status 3–4 or equivalent)
• Weight loss, reduced oral intake, increasing fatigue, reduced mobility
• Increasing symptom burden not adequately controlled
• Frequent hospital admissions or presentations
• Person or family expressing awareness that death is approaching



Find out more

[Resources about end-of-life care](#)

7.1.3 Multidisciplinary end-of-life care team

The multidisciplinary team for end-of-life care should include:

- Palliative medicine specialist
- Specialist palliative care nurse / clinical nurse practitioner
- General practitioner (GP) (with engagement and communication throughout)
- Pain specialist
- Clinical psychologist

- Social worker
- Pastoral or spiritual care worker
- Bereavement counsellor
- Cultural liaison officer or cultural expert
- Music therapist or art therapist
- Allied health (physiotherapy, occupational therapy, dietetics as relevant)
- CanTeen for children of parents with cancer.



Complete the list

The end-of-life care team for [population group] should include:

- *list additional members and / or expertise specific to this population group*



<Optional> Complete the box



End-of-life care planning checklist

- ✓ Ensure the person has an advance care directive in place
- ✓ Ensure the person has been referred to palliative care
- ✓ Discuss and document the person's preferred place of care and death
- ✓ Consider referral to home and community-based care, specialist community palliative care workers or community nursing as appropriate
- ✓ Respond to any questions or information requests for voluntary assisted dying
- ✓ End-of-life planning considerations for [population group] with cancer include:
 - *list end-of-life care considerations specific to this population group*

7.2 Delivering end-of-life care

7.2.1 Care setting

The care setting is an important consideration for both a person at the end of their life and their family.



<Optional> Complete the box



End-of-life care setting checklist

- ✓ Support care at home where this is the patient's preference and it is clinically and logistically feasible
- ✓ Ensure the transition to the chosen care setting is planned, not reactive
- ✓ Offer access to private physical spaces for gatherings as required
- ✓ Coordinate with home-based community palliative care services, residential hospice, or inpatient palliative unit as appropriate
- ✓ Actions related to the setting for end-of-life care specific to this population group include:

– list actions related to end-of-life care setting for people in this population group

7.2.2 Symptom management

Comprehensive symptom management is the core clinical work of end-of-life care.



Timely identification and management of palliative care emergencies is important for symptom management and to maintain quality of life.⁸⁵



<Optional> Complete the box

Cancer-specific OCPs include details of symptoms requiring management at end of life. Consider using the box below to outline population-specific considerations for symptom management at end of life for people with cancer

Assess and actively manage the following:

Pain	Use validated pain assessment tools and ensure regular analgesia review; refer to pain specialist if needed
Breathlessness	Optimise pharmacological and non-pharmacological management
Other physical symptoms	Assess for and manage nausea and vomiting, fatigue and weakness, mouth care and skin integrity
Anxiety and psychological distress	Screen regularly and refer to psychology or psychiatric services
Delirium	Identify and manage reversible causes; provide family support
Nutritional decline and eating difficulties	Involve a dietitian, discuss with patient and family

Considerations for symptom management for [population group] at end of life include:

- list considerations for symptom management at end of life specific to this population group.

7.2.3 Family and carer support

Carers and family members should receive support through the end-of-life period and in bereavement.



<Optional> Complete the box



Key actions to support families and carers

- ✓ Undertake a specific needs assessment for families and carers and re-evaluate it regularly.
- ✓ Ensure families and carers receive information, support and guidance about their caring role according to their needs and wishes, including practical guidance on day-to-day care and medications where care is provided at home.
- ✓ Assess for anticipatory grief and the risk of prolonged grief, and refer to psychological, social work, pastoral or bereavement support as required.
- ✓ Provide information about practical matters, including available support services, financial and legal considerations, and what to do after a death, and offer referral to bereavement support that continues after the person has died.

✓ Actions to support families and carers of [population group] at end of life include: – List actions or considerations for family and carer support specific to this population group

 Find out more
Resources about end-of-life care
<Optional> Add resources relevant to end-of-life care specific to this cancer type, including hyperlink

7.2.4 Involvement of the general practitioner

The GP often has a central role in end-of-life care, particularly when a person chooses to be cared for, and to die, at home.

Working alongside the specialist and community palliative care teams, the GP can:

- provide and coordinate symptom management
- attend the person in the community
- offer continuity of care for the person
- provides ongoing support to the family and carers, including into bereavement.

The GP should be kept fully informed and involved in major developments in the person's care. Clear communication about the person's condition, goals of care and preferred place of care and death supports a planned rather than reactive approach.

 <Optional> Complete the box
 Key actions for communicating with the GP regarding end-of-life care
✓ Keep the GP fully informed of major developments and involve them in end-of-life care planning
✓ End-of-life care planning should be discussed with the GP so that the person's needs and goals can be met in their preferred environment.
✓ Actions related to GP communication regarding end-of-life care for [population group] include: – list actions for GP communication regarding end-of-life care specific to this population group

7.3 Voluntary assisted dying

Voluntary Assisted Dying (VAD) is legally available in all Australian states and the Australian Capital Territory (ACT). There are legal requirements all medical practitioners must comply with, irrespective of personal views.

All states except Tasmania and the ACT require the medical condition to be progressive. In all states and the ACT, clinicians who have not completed mandatory VAD training must decline requests. Clinicians who are unable to provide VAD services should comply with the legal requirements in their jurisdiction, including providing information about appropriate referral pathways where required.

Clinicians may make a conscientious objection to participating in a VAD process but must be aware of their jurisdiction-specific obligations, which vary in terms of information provision and referral requirements.

VAD care navigators exist in all states and the ACT. Refer patients and families to these services for information and support.

 **Find out more**

[Resources about voluntary assisted dying](#)

Supportive care



<Optional> Complete the boxes

Use the boxes below if relevant to provide information about supportive care and communication considerations and actions specific to this population group



Supportive care considerations at end-of-life

- Emotional and psychological distress is common, including anticipatory grief, fear of death or dying, anxiety, depression and interpersonal difficulties.
- Physical symptoms and a decline in mobility or functional status are common and may affect the person's care needs and place of care.
- People may need support to appoint a substitute decision-maker and complete an advance care directive, and to manage legal and financial matters such as making a will, arranging care of dependent children, and lodging insurance, superannuation or Centrelink claims on the basis of terminal illness or permanent disability.
- Families and carers have their own needs, including practical support, respite and spiritual support.
- Cultural, spiritual and religious beliefs and practices strongly shape what matters most to a person and their family at the end of life and should be respected and accommodated.
- Supportive care considerations at end of life for [population group] include:
 - *list considerations for supportive care at end of life specific to this population group; use a new row for each consideration.*



Supportive care checklist

- ✓ Assess physical, psychological, social, spiritual and existential needs using a validated tool and document the results, reassessing as needs change
- ✓ Assess and actively manage symptoms, and refer to specialist palliative care, pain and allied health services as required
- ✓ Confirm the person has an advance care directive and substitute decision-maker in place
- ✓ Discuss and document the person's preferred place of care and place of death
- ✓ Identify and support practical, legal and financial needs, including a will, care of dependent children and insurance, superannuation or Centrelink claims
- ✓ Assess carer and family needs and offer support, respite and referral, including for anticipatory grief, bereavement and support for dependent children
- ✓ Provide referrals to appropriate sources of information, support and navigation via the [Australian Cancer Nursing and Navigation Program](#) (ACNNP):
 - Cancer Navigation Service ([Cancer Connect](#), 13 11 20 or [Cancer Hub](#))
 - Cancer Care Nursing Services ([Cancer Care Nurse Service](#) or [Prostate Cancer Specialist Nurses](#))
 - Specialist support services (via [cancer-specific non-government organisations](#))
- ✓ Provide referrals to appropriate sources of information and support for people receiving palliative care e.g. via [Palliative Care Australia](#)

✓ Supportive care actions at end of life for [population group] include: – <i>list supportive care actions at end of life specific to this population group; use a new row for each action</i>

 Communication considerations for end-of-life care
• Communication should be honest, compassionate and paced to what the person and their family wish to know, respecting their cultural, spiritual and personal beliefs.
• Conversations should not offer unreasonable hope.
• Communication considerations at end of life for [population group] include: – <i>list communication considerations at end of life specific to this population group; use a new row for each consideration</i>

 Communication checklist
✓ Check the person's understanding of their condition and prognosis and tailor conversations to what they and their family wish to know
✓ Hold honest, compassionate conversations about goals of care, palliative care and the person's preferred place of care and death, respecting cultural and spiritual needs
✓ Provide the person, family and carers with contact details for the care team and palliative care service, including after-hours contacts, and clear information about what to do as death approaches and after a death
✓ Communication actions at end of life for [population group] include: – <i>list communication actions at end of life specific to this population group; use a new row for each action</i>

 Find out more
Resources about palliative care Resources about end-of-life care Resources about advance care planning
<Optional> Add resources about support and communication at end of life specific to this population group, including hyperlink
 See also
• Supportive care (Principle) <hyperlink to be added> • Communication (Principle) <hyperlink to be added>

Contributors and reviewers

Our thanks to the following health professionals, consumer representatives, stakeholders and organisations consulted in developing this optimal care pathway.

Expert working group (this edition)

List members of working group involved

Expert working group (previous edition)

List members of working group involved in 1st edition if this is 2nd edition

Medical colleges and peak organisations invited to provide feedback

Add / remove cancer-specific colleges and organisations invited to provide feedback

Advance Care Planning Australia

Allied Health Professions Australia

Australasian Association of Nuclear Medicine Specialists

Australasian Chapter of Palliative Medicine,
Royal Australia College of Physicians

Australian and New Zealand Society of
Neuroradiology

Australian and New Zealand Society of
Palliative Care

Australian Cancer Survivorship Centre

Australian College of Nursing

Australian Medical Association

Australian Society of Medical Imaging and
Radiation Therapy

Cancer Nurses Society of Australia

Clinical Oncology Society of Australia

Interventional Radiology Society of Australasia

Medical Oncology Group of Australia

Oncology Social Workers Australia and New
Zealand

Royal Australasian College of Physicians

Royal Australasian College of Surgeons

Royal Australian and New Zealand College of
Radiologists

Royal Australian College of General
Practitioners

Royal College of Pathologists of Australasia

Find out more

Step 1: Prevention, screening and early detection

Risk reduction strategies

 Find out more
RACGP Guidelines for preventive activities in general practice Supports evidence-based screening, case finding and preventive activities in primary care (including some cancer-specific guidance) <hyperlink https://www.racgp.org.au/getattachment/9755764e-25f8-4799-bbca-29ddaf8c6d65/Guidelines-for-preventive-activities-in-general-practice.aspx >
RACGP Putting prevention into practice Guidelines for the implementation of prevention in the general practice setting Supports clear and actionable plans to improve preventative care in general practice <hyperlink https://www.racgp.org.au/getattachment/2ba9e40f-fe33-44bf-8967-8bf6f18a1c1a/Putting-prevention-into-practice-Guidelines-for-the-implementation-of-prevention-in-the-general-practice-setting.aspx >
Cancer Council Australia Causes and Prevention: Reduce your cancer risk Provides information about strategies to reduce common risks for cancer (public / community resource) <hyperlink https://www.cancer.org.au/cancer-information/causes-and-prevention >
Cancer Institute NSW: Damaging effects of vaping A plain language overview of the health risks of vaping, with cessation support resources (public / community resource) <hyperlink https://www.cancer.nsw.gov.au/prevention-and-screening/preventing-cancer/damaging-effects-of-vaping >

Genetic testing

 Find out more
eviQ Cancer Genetics: Referral Guidelines Evidence-based referral criteria to identify patients who should be referred to a familial cancer clinic for genetic counselling and risk assessment <hyperlink https://www.eviq.org.au/cancer-genetics/referral-guidelines >
eviQ Guide for Health Professionals Ordering Genetic Testing Practical guidance for clinicians on when and how to order germline genetic testing, including pre-test requirements and interpretation of results <hyperlink https://www.eviq.org.au/cancer-genetics/resources/3751-guide-for-health-professionals-ordering-genet >
eviQ Adult Risk Management Guidelines Evidence-based surveillance and risk management recommendations for adults confirmed to carry a pathogenic variant in a hereditary cancer predisposition gene <Hyperlink https://www.eviq.org.au/cancer-genetics/adult/risk-management >
eviQ Paediatric Risk Management Guidelines Evidence-based surveillance and risk management recommendations for adults confirmed to carry a pathogenic variant in a hereditary cancer predisposition gene <hyperlink https://www.eviq.org.au/cancer-genetics/paediatric/risk-management >
Selection of Germline Genetic Testing Panels in Patients With Cancer: ASCO Guideline Provides evidence-based recommendations on how to select and apply multigene panels for

germline genetic testing in people with cancer, based on personal and family history Tung <i>et al.</i> 2024 <hyperlink https://pubmed.ncbi.nlm.nih.gov/38759122/ >
Centre for Genetics Education Plain-language information about cancer risk, family history and genetic services in Australia, for both health professionals and the public <hyperlink https://www.genetics.edu.au/SitePages/Home.aspx >
Inherited Cancers Australia: Genetic Testing Consumer-focused information about accessing genetic testing in Australia, including how to navigate public and private pathways and what to expect from the process <hyperlink https://www.inheritedcancers.org.au/i-want-to-test/genetic-testing >

Step 2: Presentation, initial investigations and referral

i Find out more
National Institute for Health and Care Excellence 2015 guidelines, Suspected cancer: recognition and referral Recommendations on patient support, safety netting and the diagnostic process <hyperlink https://www.nice.org.uk/guidance/ng12/chapter/Recommendations-on-patient-support-safety-netting-and-the-diagnostic-process >

Step 3: Diagnosis, staging and treatment planning

Genomics

i Find out more
Cancer Australia: National Framework for Genomics in Cancer Control A national framework to guide policy and ensure equitable access to genomics in cancer care for all Australians affected by cancer <hyperlink https://www.canceraustralia.gov.au/key-initiatives/national-framework-genomics-cancer-control >
Omico Cancer Screening Program – CaSP Australian program for genomic profiling for patients with advanced, incurable or an earlier diagnosis of a poor prognosis cancer that aims to identify potential matches for patients to clinical trials with new targeted therapies <hyperlink https://omico.org.au/our-programs/cancer-screening-program-casp/ >
eviQ Genetic testing using cancer gene panels Guides for certain cancer types for which gene panels exist <hyperlink https://www.eviq.org.au/cancer-genetics/adult/genetic-testing-using-cancer-gene-panels >

Staging

i Find out more
Cancer Institute NSW: Understanding the stages of cancer A plain-language explanation of how cancer staging works, including the TNM system and numbered stage classifications <hyperlink: www.cancer.nsw.gov.au/understanding-cancer/stages-of-cancer >

Performance status



Find out more

European Cooperative Research Group (ECOG) Performance Status Scale A scale adopted in cancer clinical trials worldwide that measures a patient's level of functioning in terms of their ability to care for themselves, daily activity, and physical ability (walking, working, etc.) <hyperlink: <https://ecog-acrin.org/resources/ecog-performance-status/>>

Step 4: Treatment

Prehabilitation



Find out more

Agency for Clinical Innovation (ACI). Prehabilitation: Key principles for preparing patients for surgery Outlines key principles for preparing people for surgery <Hyperlink https://aci.health.nsw.gov.au/_data/assets/pdf_file/0005/743360/ACI-Prehabilitation-key-principles-for-preparing-patients-for-surgery.pdf>

Surgery



Find out more

Royal Australasian College of Surgeons Resources on surgical oncology standards, position papers, and minimum standards for training and credentialing of cancer surgeons. <Hyperlink <https://www.surgeons.org/Resources/interest-groups-sections/surgical-oncology>>

Radiation therapy



Find out more

eviQ Radiation Oncology Protocols Current evidence-based, peer-maintained radiation therapy treatment protocols covering all major cancer types <hyperlink <https://www.eviq.org.au/radiation-oncology>>

RANZCR Standards of Practice for AI in Radiation Oncology RANZCR standards guiding the safe and appropriate use of artificial intelligence tools in radiation oncology practice in Australia and New Zealand <hyperlink <https://www.ranzcr.com/document/standards-of-practice-for-ai-in-radiation-oncology/>>

Systemic therapies



Find out more

eviQ Medical Oncology Protocols Current evidence-based, peer-maintained systemic therapy treatment protocols covering all major cancer types, developed by Cancer Institute NSW for use by Australian medical oncology teams. <hyperlink <https://www.eviq.org.au/medical-oncology>>

eviQ Calculators	A suite of clinical calculators supporting safe chemotherapy prescribing in the Australian context, including tools for body surface area, creatinine clearance, and dose adjustment <hyperlink https://www.eviq.org.au/clinical-resources/evi-q-calculators >
eviQ ADDIKD: Cancer Drug Dosing in Kidney Dysfunction Guideline	An Australian evidence-based guideline supporting clinicians to safely adjust anti-cancer drug doses for patients with renal impairment across a range of cancer treatment regimens <hyperlink https://www.eviq.org.au/clinical-resources/addikd-guideline >
eviQ Administration of Anti-Cancer Drugs	Practical guidance and safety resources for the preparation and administration of anti-cancer drugs in the Australian clinical setting, including handling, extravasation management and central venous access. <hyperlink https://www.eviq.org.au/clinical-resources/administration-of-anti-cancer-drugs >
European Society of Medical Oncology Magnitude of Clinical Benefit Scale	Tool to facilitate decision-making about anti-cancer therapies integrating quality of life and survival considerations; available for solid tumours and haematological malignancies <hyperlink https://www.esmo.org/guidelines/esmo-mcbs >
Cancer Nurses Society of Australia (CNSA) Vascular Access Devices: Evidence-Based Clinical Practice Guidelines	Australian evidence-based guidelines from CNSA for the safe and effective management of vascular access devices for people with cancer. <hyperlink https://www.cnsa.org.au/implement/vascular-access-devices.html >
Venous Thromboembolism in Cancer Patients: ESMO Clinical Practice Guideline	An ESMO clinical practice guideline providing evidence-based recommendations for the prevention, diagnosis and management of venous thromboembolism in people with cancer Falanga <i>et al.</i> (2023) <hyperlink https://pubmed.ncbi.nlm.nih.gov/36638869/ >
Cancer-Associated Venous Thrombosis in Adults: A British Society for Haematology Guideline (2nd edition)	A UK clinical guideline covering the prevention and treatment of cancer-associated venous thrombosis, including recommendations on anticoagulant selection, duration of therapy and special populations Alikhan <i>et al.</i> (2024) <hyperlink https://pubmed.ncbi.nlm.nih.gov/38664942/ >
eviQ Haematology Protocols	Current evidence-based protocols for treatment of haematological malignancies including leukaemias, lymphomas and myeloma, developed by Cancer Institute NSW for use by Australian haematology teams. <hyperlink https://www.eviq.org.au/haematology >
eviQ Cellular Therapy Protocols	Evidence-based protocols for cellular therapies used in cancer treatment in Australia, including CAR T-cell therapies and blood and marrow transplantation. <hyperlink https://www.eviq.org.au/cellular-therapies >

Nuclear medicine

 Find out more	
Australasian Association of Nuclear Medicine Specialists. Referrer's Guide to nuclear medicine procedures including positron emission tomography procedures.	An overview of nuclear medicine covering diagnostic and therapeutic applications including PET, SPECT and radiopharmaceutical therapies <hyperlink https://aanms.org.au/what-is-nuclear-medicine/ >

Clinical trials

 Find out more
Cancer Australia Australian Cancer Trials The Australian Cancer Trials database provides up-to-date information on active clinical trials spanning various cancer types, serving as a valuable resource for both clinicians and patients seeking appropriate trial opportunities. <hyperlink https://australiancancertrials.gov.au/>
Collaborative clinical trials groups undertake clinical trials for a range of different cancer types <hyperlink https://www.canceraustralia.gov.au/research-grants/support-cancer-clinical-trials/multi-site-collaborative-cancer-clinical-trials>
Australian Commission on Safety and Quality in Healthcare National Clinical Trials Governance Framework and user guide <hyperlink https://www.safetyandquality.gov.au/national-standards/national-clinical-trials-governance-framework>
Cancer Australia Understanding Clinical Trials Information about how cancer clinical trials are run in Australia with links to relevant policies <hyperlink https://www.canceraustralia.gov.au/understanding-clinical-trials>
Australian Clinical Trials Talking to your patients about clinical trials Good practice principles for talking to patients about clinical trials <https://www.australianclinicaltrials.gov.au/healthcare-providers/talking-to-your-patients>
Teletrials and decentralised trials
Australian Tele-Trial program A national program supporting tele-trials with regional coordinating centres that support clinicians, researchers and sponsors set up and run local trials <hyperlink https://www.australianclinicaltrials.gov.au/healthcare-providers/talking-to-your-patients>
Regional trial networks are available supporting clinical trials in regional parts of Australia: • NSW Regional Cancer Research Network <hyperlink https://nswregionalcrcn.com.au/home> • Regional Trials Network Victoria <hyperlink https://regionaltrialsnetwork.org/>
Interpreters and clinical trials
eviQ Education Training for healthcare interpreters on cancer clinical trials Education course for healthcare interpreters involved in talking to patients about cancer clinical trials <hyperlink https://education.eviq.org.au/clinical-and-community-practice/training-for-health-care-interpreters-on-cancer-clinical-trials>
Research and clinical trials for Aboriginal and Torres Strait Islander peoples
NHMRC Ethical conduct in research with Aboriginal and Torres Strait Islander Peoples and communities: Guidelines for researchers and stakeholders National guidelines for research involving Aboriginal and Torres Strait Islander peoples <hyperlink: https://www.nhmrc.gov.au/about-us/publications/ethical-conduct-research-aboriginal-and-torres-strait-islander-peoples-and-communities>
Menzies School of Health Research Aboriginal and Torres Strait Islander People with Cancer - Clinical Trial Access Initiative: Consultancy Report for ReViTALISE Project Guidance on clinical trial access for Aboriginal and Torres Strait Islander people with cancer, developed for the ReViTALISE project <hyperlink: https://www.menzies.edu.au/icms_docs/332413_Aboriginal_and_Torres_Strait_Islander_People_with_Cancer_Clinical_Trial_Access_Initiative_Consultancy_Report_for_ReViTALISE_Project_July2022.pdf>

Interventional radiology



Find out more

RANZCR Standards of Practice for Interventional Radiology and Interventional Neuroradiology Standards for the safe and effective preparation, delivery and follow-up of interventional radiology and interventional neuroradiology procedures <hyperlink <https://www.ranzcr.com/wp-content/uploads/2022/07/20220610-Standards-of-Practice-for-Interventional-Radiology-and-Interventional-Neuroradiology.pdf>>

Complementary therapies



Find out more

Cancer Australia Complementary Therapies hub A resource explaining the role of complementary therapies alongside conventional cancer treatment, including evidence-based therapies and guidance for health professionals and patients on safe and informed use. <Hyperlink <https://www.canceraustralia.gov.au/impacted-cancer/treatment/complementary-therapies>>

Step 5: Care after initial treatment and recovery

GP Chronic Condition Management Plan



Find out more

Services Australia GP Chronic Condition Management Plan (GPCCMP) Information on the steps to take to create a GPCCMP <hyperlink <https://www.servicesaustralia.gov.au/gp-chronic-condition-management-plan?context=20>>

Step 6: Management of residual, recurrent and metastatic disease

Palliative care



Find out more

Palliative Care Australia <Hyperlink to <https://palliativecare.org.au>> National peak body with plain-language information on palliative care, finding services and what to expect.

palliAGED A palliative care evidence and practice resource for the Australian aged care sector <hyperlink to <https://www.palliaged.com.au>>

CareSearch A national website connecting healthcare professionals with evidence-based palliative care information <hyperlink to <https://www.caresearch.com.au>>

Australian Department of Health and Aged Care: Palliative Care A central hub for national palliative care policy, programs and resources, including information on the National Palliative Care Strategy and funding initiatives <hyperlink <https://www.health.gov.au/health-topics/palliative-care>>

Guidelines and frameworks

Palliative Care Australia: National Palliative Care Standards A nationally consistent framework for safe, effective palliative care, with one set for specialist services and one for all health

professionals and aged care services <hyperlink to https://palliativecare.org.au/national-palliative-care-standards >
CareSearch: Palliative Care Guidelines A curated collection of Australian and international palliative care guidelines, maintained by CareSearch, providing health professionals with access to evidence-based guidance across all aspects of palliative care practice <hyperlink https://www.caresearch.com.au/Evidence/Evidence-Collections/Palliative-Care-Guidelines >
Australian Institute of Health and Welfare: National Palliative Care Measures A national reporting framework and data collection developed by the AIHW to monitor the quality, access and outcomes of palliative care services across Australia <hyperlink https://www.aihw.gov.au/reports/palliative-care-services/national-palliative-care-measures/contents/measures >
Reports
Australian Institute of Health and Welfare: Palliative Care and Health Service Use for People with Life-Limiting Conditions A report examining patterns of health service use, including hospital admissions and palliative care service access <hyperlink https://www.aihw.gov.au/reports/palliative-care-services/health-service-use-people-life-limiting-conditions/contents/introduction >
Communication tools
Queensland Health: Sad news, sorry business Guidance on caring for Aboriginal and Torres Strait Islander people around death and dying in a culturally appropriate way <hyperlink https://www.health.qld.gov.au/_data/assets/pdf_file/0023/151736/sorry_business.pdf >
Dying to Talk An Australian Government-funded program supporting Australians to have conversations about dying, death and end-of-life care preferences, with resources for individuals, families and health professionals <hyperlink https://www.dyingtotalk.org.au >
Caring@home: Prompts at End of Life Planning (PELP) Framework An Australian Government-funded framework providing health professionals with structured prompts to initiate and guide end-of-life planning conversations with patients and families in community and home care settings <hyperlink https://www.caringathomeproject.com.au/for-health-professionals/pelp-framework >

Advance care planning

i Find out more
Advance Care Planning Australia The national resource hub for advance care planning in Australia, providing information, tools and templates for health professionals, individuals and carers to support informed decision-making about future care. Contact the Advance Care Planning National Phone Advisory Service on 1300 208 582. <hyperlink https://www.advancecareplanning.org.au >

Step 7: End-of-life care

i Find out more
Prognostic evaluation in patients with advanced cancer in the last months of life: ESMO Clinical Practice Guideline An ESMO Clinical Practice Guideline providing key recommendations for using prognostic estimates in advanced cancer Stone et al. 2023 <hyperlink https://pubmed.ncbi.nlm.nih.gov/37087198/ >

Care of the adult cancer patient at the end of life: ESMO Clinical Practice Guidelines An ESMO Clinical Practice Guideline providing key recommendations for end-of-life care for patients with advanced cancer Crawford et al. 2021 <hyperlink https://pubmed.ncbi.nlm.nih.gov/34474810/ >
ELDAC (End of Life Directions for Aged Care) A national advisory service offering information, practical tools and toolkits on palliative care and advance care planning <hyperlink https://www.eldac.com.au >
End of life Essentials Free peer-reviewed online education modules on end-of-life care in acute hospitals for doctors, nurses and allied health <hyperlink https://www.endoflifeessentials.com.au >
End of Life Law for Clinicians (ELLC) Free online training modules for health professionals on the law on end-of-life decision-making, with CPD hours available <hyperlink https://ellic.edu.au >
Queensland Health: Sad news, sorry business Guidance on caring for Aboriginal and Torres Strait Islander people around death and dying in a culturally appropriate way <hyperlink https://www.health.qld.gov.au/_data/assets/pdf_file/0023/151736/sorry_business.pdf >
Dying to Talk An Australian Government-funded program supporting Australians to have conversations about dying, death and end-of-life care preferences, with resources for individuals, families and health professionals <hyperlink https://www.dyingtotalk.org.au >
CareSearch Clinical Evidence Summaries. Palliative Care Emergencies Summary of evidence and associated tools for management of palliative care emergencies <hyperlink https://www.caresearch.com.au/evidence/clinical-evidence-summaries/palliative-care-emergencies/ >

Voluntary assisted dying

i Find out more
Go Gentle Australia An Australian advocacy organisation providing information and resources on voluntary assisted dying for patients, families and health professionals, including state-by-state guidance on VAD legislation and eligibility <hyperlink https://www.gogentleaustralia.org.au >

Supportive care

Cancer support services

i Find out more
Cancer Connect A free national information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink https://connect.cancer.org.au/ >
Cancer Council Australia Helpline (13 11 20) A free national telephone information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink https://www.cancer.org.au/support-and-services/support-services/13-11-20 >
CanTeen Cancer Hub A free service for families impacted by cancer to access practical and emotional support services <hyperlink https://cancerhub.org.au/ >
McGrath Foundation Cancer Nurses McGrath Cancer Care Nurses support people and their families experiencing cancer throughout treatment, for free and without a referral <hyperlink https://www.mcgrathfoundation.com.au/about/our-nurses/ >

NGO Specialist support services Non-government organisations funded through the ACNRP to provide information, support and advice to people with particular cancer types <hyperlink <https://www.health.gov.au/our-work/australian-cancer-nursing-and-navigation-program>>

Practical and social support

 Find out more
Health Direct Information about Patient Assisted Travel Schemes for people who may need to travel for treatment <hyperlink https://www.healthdirect.gov.au/travelling-to-your-healthcare-appointment >
Translating and Interpreting Service Language services for people with limited English proficiency and the organisations that support them <hyperlink: https://www.tisnational.gov.au/ >
Cancer Council Financial costs and assistance <hyperlink https://www.cancer.org.au/support-and-services/financial-assistance >
Gather My Crew Australian charity providing tools, education and guidance to help people support each other <hyperlink https://www.gathermycrew.org.au/ >
Live Work Cancer <hyperlink: https://liveworkcancer.org.au/ >

Supportive care screening tools

 Find out more
NCCN Distress Thermometer and Problem Checklist A validated screening tool that helps clinicians quickly screen people for psychological distress. <hyperlink www.nccn.org/patients/guidelines/content/PDF/distress-patient.pdf >
COSA-endorsed MASCC COG-IMPACT Tool A COSA-endorsed self-report tool for assessing the difficulties and unmet supportive care needs arising from cancer-related cognitive impairment, for use in both clinical and research settings <hyperlink https://www.cosa.org.au/advocacy/endorsed-publications/the-mascc-cog-impact/ >
Cancer Survivors' Unmet Needs Measure (CaSUN) A validated 35-item self-report tool for assessing the unmet supportive care needs of cancer survivors <hyperlink https://pubmed.ncbi.nlm.nih.gov/17177268/ >
WeCan tool Based on the National Comprehensive Cancer Needs Distress Thermometer and Problem List helps people to identify their level of concern or worry and the causes of these concerns or worries <hyperlink https://wecan.org.au/assess-your-needs/ >
This is me A tool to help people gather and write down important information to share with their cancer team; designed as part of the OlderCan resource it can be used by people of any age <hyperlink https://wecan.org.au/oldercan/cancer-diagnosis/this-is-me/ >
HEEADSS assessment Health and psychosocial health assessment for adolescents and young adults <hyperlink https://www.racp.edu.au/docs/default-source/advocacy-library/routine-adolescent-psychosocial-health-assessment.pdf >
CanTeen Adolescent and Young Adult Screening Tools Included in the Adolescent and Young Adult Oncology Psychosocial Care Manual <hyperlink https://www.canteen.org.au/sites/default/files/2021-11/CanteenYCS_AYAOncoPsychosocialCareManual.pdf >

Supportive Care Needs Assessment Tool for Indigenous People (SCNAT-IP) An evidence based supportive care needs assessment tool which accommodates the language, customs and culture-specific needs of Indigenous people with cancer <hyperlink <https://www.scnatip.org/>>

Malnutrition screening tool <hyperlink <https://www.petermac.org/patients-and-carers/health-services-for-cancer-patients/allied-health-services/nutrition-and-dietetics/malnutrition-screening-tool>>

eviQ Frailty assessment tools Information about a range of tools available to assess frailty in older people <hyperlink <https://www.eviq.org.au/clinical-resources/older-people-with-cancer/4614-frailty-and-cancer>>

Supportive care and communication about cancer risk

Find out more

Quitline A free, confidential telephone counselling service using evidence-based behaviour change techniques to help people plan and sustain a quit attempt, with referral available online or by phone (13 78 48) <Hyperlink <https://www.quit.org.au>>

Inherited Cancers Australia: Genetic Testing Consumer-focused information about accessing genetic testing in Australia, including how to navigate public and private pathways and what to expect from the process <hyperlink <https://www.inheritedcancers.org.au/i-want-to-test/genetic-testing>>

Fertility

Find out more

COSA Fertility Preservation for People with Cancer: Clinical Guidelines Australian evidence-based guidelines to assist health professionals in discussing fertility preservation options with people of reproductive age prior to cancer treatment. <Hyperlink <https://www.cancer.org.au/health-professionals/clinical-practice-guidelines/cancer-fertility-preservation>>

Practice Committee of the American Society for Reproductive Medicine: Fertility drugs and cancer A US guideline reviewing the evidence on the safety of fertility medications in the context of cancer risk, for use in clinical decision-making around fertility treatment for people with or at risk of cancer. <Hyperlink <https://pubmed.ncbi.nlm.nih.gov/38703170/>>

Prehabilitation and rehabilitation

Find out more

Agency for Clinical Innovation (ACI). Prehabilitation: Key principles for preparing patients for surgery Outlines key principles for preparing people for surgery <Hyperlink https://aci.health.nsw.gov.au/_data/assets/pdf_file/0005/743360/ACI-Prehabilitation-key-principles-for-preparing-patients-for-surgery.pdf>

Agency for Clinical Innovation Rehabilitation Model of Care A framework guiding the delivery of equitable, person-centred, multidisciplinary rehabilitation services across care settings, supporting health planners and clinicians to optimise patient outcomes and system efficiency<hyperlink <https://aci.health.nsw.gov.au/projects/rehab-model-of-care>>

Survivorship care

 Find out more
<u>MyCarePlan</u> A free online tool from the Australian Cancer Survivorship Centre that helps people with cancer create a personalised survivorship care plan <hyperlink https://www.mycareplan.org.au>
<u>Australian Cancer Survivorship Centre</u> A national centre providing expertise, information, education and support to enable optimal survivorship care for people affected by cancer in Australia; includes fact sheets for follow-up care for some cancer types and templates for survivorship care plans <hyperlink https://www.petermac.org/cancersurvivorship>
<u>Cancer Australia: Principles of Cancer Survivorship (2026)</u> Cancer Australia's national framework outlining the principles that should guide survivorship care for all people with cancer <hyperlink https://www.canceraustralia.gov.au/publications-and-resources/cancer-australia-publications/principles-cancer-survivorship>
<u>Cancer Australia: Shared Cancer Follow-up and Survivorship Care</u> A Cancer Australia resource outlining the evidence and principles for shared follow-up care between GPs and specialist cancer services <hyperlink https://www.canceraustralia.gov.au/clinical-best-practice/shared-follow-care>
<u>COSA Position Statement on Cancer Survivorship Care</u> A Clinical Oncology Society of Australia (COSA) position statement setting out the essential components of high-quality cancer survivorship care in Australia <hyperlink https://www1.racgp.org.au/ajgp/2019/december/clinical-oncology-society-of-australia-position-st>
<u>eviQ Education: Cancer Survivorship</u> A free online course from Cancer Institute NSW providing health professionals with a foundational understanding of survivorship care principles <hyperlink https://education.eviq.org.au/courses/cancer-survivorship>
<u>South Australian Cancer Survivorship Framework</u> A state-wide framework from SA Health setting out a model for the delivery of cancer survivorship care in South Australia <hyperlink https://www.sahealth.sa.gov.au/wps/wcm/connect/public+content/sa+health+internet/about+us/publications+and+resources/plans/south+australian+survivorship+framework>
<u>Peter MacCallum Cancer Centre: Common Survivorship Issues Directory</u> A practical directory from Peter Mac providing health professionals and people with cancer with information on common physical and psychosocial issues arising after cancer treatment <hyperlink https://www.petermac.org/patients-and-carers/support-and-wellbeing/living-with-and-beyond-cancer/common-survivorship-issues-directory>
<u>RACGP Position Statement: Cancer Survivorship Shared Care (2024)</u> A statement calling for a collaborative shared care model between GPs, specialist cancer services and patients for cancer survivorship <hyperlink https://www.racgp.org.au/advocacy/position-statements/view-all-position-statements/clinical-and-practice-management/cancer-shared-care>
<u>Services Australia: Mental Health Care and Medicare</u> An overview of Medicare-subsidised mental health services available to Australians <hyperlink https://www.servicesaustralia.gov.au/mental-health-care-and-medicare?context=60092>
<u>CancerChoices: Exploring What Matters Now</u> A reflective resource from CancerChoices supporting people with cancer to identify their values, priorities and goals as a guide to decision-making and living well after diagnosis <hyperlink https://cancerchoices.org/healing-practice/exploring-what-matters-now/>

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